

Westborough Behavioral Health Hospital

DISCHARGE MEDICATION SUMMARY FOR PATIENT

Patient Name: Flaherty, Dallas G

DOB: 11/03/1994 Room-Bed: 2210b

Admit Date/Time: 7/22/25 17:35

Ht: 70.00cm

Wt: 0.00kg

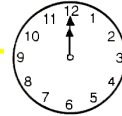
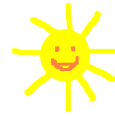
BMI: 0.00

BSA: 0.00

Regular Pharmacy: WALGREENS DRUG STORE #05755

Allergies: metoclopramide, NO FOOD ALLERGIES

Medications to take after Discharge



Medication	Dose	Route	Freq	Morning	Noon	Evening	Bedtime
CARDIOVASCULAR AGENTS							
amLODIPine 10 mg TAB (Norvasc) Indication: Essential (primary) hypertension Last Dose: 7/29/25 8:21 am	10 mg	Oral	ONCE A DAY	✓			
hydroCHLORothiazide 25 mg TAB (Oretic) Indication: Essential (primary) hypertension Last Dose: 7/29/25 8:21 am	25 mg	Oral	ONCE A DAY	✓			
CENTRAL NERVOUS SYSTEM AGENTS							
busPIRone 10 mg TAB (BuSpar) Indication: Generalized anxiety disorder Last Dose: 7/29/25 8:21 am	10 mg	Oral	TWICE A DAY	✓			✓
divalproex sodium ERT 500 mg ERT (Depakote ER) Indication: Epilepsy,unspecified,not intractable,without status epilepticus Last Dose: 7/29/25 8:21 am	1,000 mg	Oral	ONCE A DAY	✓			
acetaminophen 325 mg TAB (Tylenol) Indication: pain Last Dose:	650 mg	Oral	EVERY 6 HOURS AS NEEDED				
GASTROINTESTINAL AGENTS							
omeprazole 20 mg DRC (PriLOSEC) Patient Note: SALAD Indication: GERD	20 mg	Oral	ONCE A DAY	✓			

Do NOT Scan to Pharmacy

ACCT #: 2501689

MR #: 109931

Admit Date: 7/22/25 17:35

Physician: Okorafor, Ayodeji E, DNP

DOB: 11/03/1994

AGE: 31 years

SEX: M

printed 3/31/26 11:12

Transition Record - Part 3

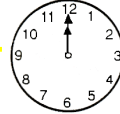
Flaherty, Dallas G

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Westborough Behavioral Health Hospital

DISCHARGE MEDICATION SUMMARY FOR PATIENT

Patient Name: Flaherty, Dallas G



Medication	Dose	Route	Freq	Morning	Noon	Evening	Bedtime
Last Dose: 7/29/25 8:21 am							
MISCELLANEOUS AGENTS							
nicotine 2 mg GUM (Nicorette) Indication: Smoking Cessation Last Dose: 7/28/25 6:06 pm	4 mg	Oral Transmucosal	EVERY 2 HOURS AS NEEDED				
nicotine 21 mg/24 hr 21 mg/24 hr TERF (nicotine patch 21mg) Indication: Nicotine Dependence Last Dose:	1 PATCH	Transdermal	ONCE A DAY AS NEEDED				
PSYCHOTHERAPEUTIC AGENTS							
FLUoxetine 20 mg CAP (PROzac) Indication: Anxiety, current episode manic without psychotic features, severe Last Dose: 7/29/25 8:21 am	20 mg	Oral	ONCE A DAY	✓			
UNKNOWN							
LORazepam 0.5 mg TAB (Ativan) Patient Note: SALAD Indication: Generalized anxiety disorder Last Dose: 7/29/25 10:13 am	0.5 mg	Oral	TWICE A DAY AS NEEDED				

Keep a list of your Medications and update with any changes including non prescription products. Tell all of your physicians about changes at each visit and carry a copy with you at all times in case of an emergency situations. Always take medications according to your physicians instructions. Discard Medications that have expired.

Do NOT Scan to Pharmacy

printed 3/31/26 11:12

Transition Record - Part 3

ACCT #: 2501689

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Physician: Okorafor, Ayodeji E, DNP

DOB: 11/03/1994

AGE: 31 years

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Flaherty, Dallas G

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Westborough Behavioral Health Hospital

DISCHARGE MEDICATION SUMMARY FOR PATIENT

Patient Name: Flaherty, Dallas G

DO NOT CONTINUE THESE MEDICATIONS AT HOME

Medication	Dose	Route	Frequency
FLUoxetine 40 mg CAP PROzac	40 mg	Oral	ONCE A DAY
divalproex sodium ERT 500 mg ERT Depakote ER Notes:Once-Daily	500 mg	Oral	ONCE A DAY

DO NOT CONTINUE

Do NOT Scan to Pharmacy

printed 3/31/26 11:12
Transition Record - Part 3

ACCT #: 2501689

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Admit Date: 7/22/25 17:35

Physician: Okorafor, Ayodeji E, DNP

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Flaherty, Dallas G

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Westborough Behavioral Health Hospital

DISCHARGE MEDICATION SUMMARY FOR PATIENT

Patient Name: Flaherty, Dallas G

THESE DISCHARGE PRESCRIPTIONS e-PRESCRIBED TO:

WALGREENS DRUG STORE #05755
413 WASHINGTON ST
STOUGHTON, MA 020724210
Phone Number: 7813445600
03/31/2026 11:12 am

Take this form to your pharmacy to pick up your medications.

All these medications are listed on the front sheet under Medications to take after Discharge.

Medication	Dose	Route	Frequency	Quantity	Refills
busPIRone <i>BuSpar</i>	10 mg	Oral	TWICE A DAY	14	0
amLODIPine <i>Norvasc</i>	10 mg	Oral	ONCE A DAY	7	0
hydroCHLORothiazide <i>Oretic</i>	25 mg	Oral	ONCE A DAY	7	0
omeprazole <i>PriLOSEC</i>	20 mg	Oral	ONCE A DAY	7	0
FLUoxetine <i>PROzac</i>	20 mg	Oral	ONCE A DAY	7	0
divalproex sodium ERT <i>Depakote ER</i>	1,000 mg	Oral	ONCE A DAY	14	0

Patient/Guardian Signature Date/Time

Nurse Signature Date/Time

Do NOT Scan to Pharmacy

printed 3/31/26 11:12
Transition Record - Part 3

ACCT #: 2501689

MR #: 109931

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Physician: Okorafor, Ayodeji E, DNP

DOB: 11/03/1994

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Flaherty, Dallas G

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Clinical Progress Note

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/26/2025 9:21:00AM

Date/Time

7/26/2025 09:21

Treatments

Problem: Danger To Others r/t Physical Aggression, Homicidal (HI statements towards parents, property destruction,)

Problem: Danger to Self (drinking -intoxication, punching objects)

Problem: Altered Cardiac Output - Hypertension/Hypotension (Medication regimen, Verbal report of history of Hypertension)

Problem: Nicotine Replacement/Smoking Cessation (Use of nicotine patch or gum per facility protocol, History of excessive nicotine use)

Problem: Seizures (Alteration In Neurologic Homeostasis) (History of seizures. Last seizure in 2002 per pt. Pt noncompliant with medications, including seizure prevention meds.)

Summary

patient observed on the milieu in behavioral control , participating in groups but observed to become easily agitated on the phone conversation- mood labile at times. continue to monitor for any HI or aggression.

Staff Signature

e-Signed by Campbell, Deanna at 07/26/2025 09:22

Discharge Aftercare Plan

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/28/2025 2:01:02PM**

Name of Person Receiving Instructions	Dallas F
Relationship	Patient
Discharge Status	Completed Treatment Routine
Will be Returning to Work/School	N/A
Substance Abuse or Tobacco Cessation Treatment Offered	N/A
Does Patient Have Advanced Directive?	N/A
Other Instructions	Patient will discharge to mother's home via lyft 61 Drake Ave Stoughton, MA @ 2pm

Missing Appointments:

If you are unable to keep your appointment, please call the appointment location at least 24 hours in advance, if possible, so you can reschedule. It is important that you maintain contact with your physician(s) and/or therapist after discharge.

* If discharged to a home health agency (HHA), skilled nursing facility (SNF), inpatient rehab facility (IRF), or long-term care hospital (LTCH), the patient was provided Key Performance Data, including Quality Metrics to assist with discharge planning process.

Staff Initials	MB
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CRISIS EMERGENCY CONTACT INFORMATION:

The Following are Emergency Numbers I Can Call if I Need Help:

911

988 - Suicide Hotline, Call or Text

508-329-6300 - Westborough Behavioral HealthCare Hospital

Discharge Aftercare Plan

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/28/2025 2:01:02PM

Discharge Meds:

Description	Brand	Dose	Route	Frequency	PRN
amLODIPine Oral 10 mg TAB Essential (primary) -----	Norvasc	10 mg	PO	DAILY	---
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
divalproex sodium Oral 500 mg ERT Epilepsy, unspecified, not (Once-Daily) intractable, without status	Depakote	1,000 mg	PO	DAILY	
epilepticus					
FLUoxetine Oral 20 mg CAP Anxiety, current episode	PROzac	20 mg	PO	DAILY	
without psychotic					manic
features, severe					
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY	
hypertension					
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P
disorder					
Notes: SALAD					
nicotine Oral Transmucosal 2 mg Smoking Cessation	Nicorette	4 mg	OM	Q2H	P
GUM					
nicotine 21 mg/24 hr Transdermal Nicotine Dependence	*nicotine	1	TD	DAILY	P
TERF	patch	PATCH			
omeprazole Oral 20 mg DRC	PriLOSEC	20 mg	PO	DAILY	GERD
Notes: SALAD					
acetaminophen Oral 325 mg TAB	*Tylenol	650 mg	PO	Q6H	P pain

Medication Orders:

Description	Brand	Dose	Route	Frequency	PRN
Indication					
-----	-----	-----	-----	-----	---

amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY	
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
cloNIDine (Catapres) Oral 0.1 mg	Catapres	0.1 mg	PO	TID	P

Discharge Aftercare Plan

Flaherty, Dallas G

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DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/28/2025 2:01:02PM

Elevated BP above 140/90.

Notes: Hold if HR < 60 OR BP < 90/60 or if symptomatic

SALAD

TAB

divalproex sodium Oral 500 mg ERT	Depakote	1,000	PO	DAILY
Epilepsy, unspecified, not (Once-Daily)	ER	mg		
intractable, without status				

epilepticus

Notes: Once-Daily

FLUoxetine Oral 20 mg CAP	PROzac	20 mg	PO	DAILY
Anxiety, current episode				

manic

without psychotic

features, severe

hydrochlorothiazide Oral 25 mg TAB	Oretic	25 mg	PO	DAILY
Essential (primary)				

hypertension

LORazepam Oral 0.5 mg TAB	Ativan	0.5 mg	PO	BID	P
Generalized anxiety					

disorder

Notes: SALAD

nicotine Oral Transmucosal 2 mg	Nicorette	4 mg	OM	Q2H	P
Smoking Cessation					

GUM

nicotine 21 mg/24 hr Transdermal	*nicotine	1	TD	DAILY	P
Nicotine Dependence					

TERF

patch
21mg

omeprazole Oral 20 mg DRC	PriLOSEC	20 mg	PO	DAILY	GERD
Notes: SALAD					

acetaminophen Oral 325 mg TAB	*Tylenol	650 mg	PO	Q6H	P	pain
Notes: NTE 3000 mg/day						

MEDICATION REMINDERS

DO NOT DRINK ALCOHOLIC BEVERAGES OR TAKE STREET DRUGS OF ANY KIND WHILE ON PSYCHIATRIC MEDICATIONS!

Take Medications as they are prescribed at discharge. Do not change the dosage or time, unless directed by your physician.

For adolescents, medication should be kept out of reach and in a secure place.

Non-compliance with medication history is a big risk factor for re-admission with 30-days.

Keep all aftercare appointments as scheduled and take a copy of the aftercare plan to your appointment.

Discharge Aftercare Plan

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time:

Allergy Comments	Type	Reaction	Severity	Date	Code	System
metoclopramide BRUGID--	-Drug Allergy	-----	-----	----	----	d00298-----MULTUM
NO FOOD ALLERGIES	Drug Allergy					

Continuity of Lab / X-rays / Tests
Results and Contact Information

There were no lab/test results pending at the time of discharge

Labs pending at the time of
discharge

No

In case of a medical emergency, information will be provided to medical professionals by calling 508-329-6300 and asking for the Nursing Supervisor.

I do not have access to any
prescription medications, drugs,
alcohol or drug paraphernalia, guns,
sharp utensils or any other weapons /
items that I would use with lethal
intent or intent to harm self or others.

Yes

I have received a copy of my
Aftercare Discharge Instructions and
Individual Wellness Recovery Plan
and I understand the instructions
provided and all my questions have
been answered.

Yes

I authorize the release of these
Aftercare Discharge Instructions to
the providers listed on the first page,
to assist in continuity of care.

Yes

I understand these records may include drug/alcohol/mental health/communicable disease-related information. I understand that information released could contain reference to results of HIV antibody testing. A photocopy of this authorization should be considered as valid as the original.

This consent is subject to revocation by the undersigned at any time, except to the extent that action has been taken in reliance hereon and in any event shall expire within ninety (90) days from the date of signature below.

The information being authorized to release is being disclosed to you from records protected by the Federal Confidentiality Rules (42CFR Part 2). A general authorization for the release of medical or other information is not sufficient for this purpose .

The information to be released is PRIVILEGED and CONFIDENTIAL and is intended ONLY for the use of the recipient(s) named above.

Status at Discharge

Verbalized ready for discharge

Mode of transport

Lyft

Accompanied by

Self

Discharge Aftercare Plan

Flaherty, Dallas G

MR#: **109931**

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DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/28/2025 2:01:02PM**

Accompanied by Name **self**

Signatures

The discharge instructions (pages 1 and 2) have been explained to me and my questions have been answered. I understand and agree with the instructions.

Patient Signature **Verbal Consent obtained from Flaherty, Dallas G at 07/29/2025 12:29**

Case Manager Signature **e-Signed by Bard, Meghan, COTA/L at 07/29/2025 12:30**

Nurse Signature **e-Signed by Nalunkuma, Faridah, RN at 07/29/2025 13:36**

Follow-Up Care/Referrals

Row	Resource/Facility	Provider Name	Appointment Date/Time	Address	Contact Number/Fax/Email	Reason
1	Cape Behavioral Health Center (North East)	*referral initiated*	tbd	340 Gifford St Falmouth, MA 02540		Therapy
2	Cape Behavioral Health Center (North East)	Alanya Bouchard	8/1 @ 2pm TELEHEALTH	340 Gifford St Falmouth, MA 02540		Psychiatry

Discharge Summary

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/29/2025 10:24:14AM

Initial

Date/Time:

7/29/2025 00:00

Initial

Diagnosis Code	System	Class	Type	Priority	Date
Bipolar disorder, current		Psychiatric	Provisional	Primary Diagnosis	
F31.13	ICD-10				
episode manic without			1		
psychotic features, severe					
Autistic disorder		Psychiatric	Working		
F84.0	ICD-10				
Generalized anxiety disorder		Psychiatric	Working		
F41.1	ICD-10				
Personal history of traumatic		Medical	Working		
Z87.820	ICD-10				
brain injury					
Alcohol abuse, uncomplicated			Working		
F10.10	ICD-10				
Attention-deficit			Working		
F90.9	ICD-10				
hyperactivity disorder,					
unspecified type					
Cannabis abuse, uncomplicated			Working		
F12.10	ICD-10				
Epilepsy, unspecified, not			Working		
G40.909	ICD-10				
intractable, without status					
epilepticus					
Essential (primary)			Working		
I10	ICD-10				
hypertension					
Nicotine dependence,			Working		
F17.200	ICD-10				
unspecified, uncomplicated					
Pervasive developmental			Working		
F84.9	ICD-10				
disorder, unspecified					
Presence of other specified			Working		
Z97.8	ICD-10				
devices					

Discharge Summary

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/29/2025 10:24:14AM

Reason for Admission

Dallas is a 30 y.o. male presenting to WBHH from Good Samaritan. Pt was sectioned by police, for HI (stating he was going to kill parents/ hit father) after an argument with his parents. PPHx of bipolar d/o, ASD, GAD, PDD nos, and ADHD. PMH of seizure d/o, TBI (brain bleed/ lack of oxygen at birth) and HTN. Allergy to Reglan. In-house urine tox positive for benzos and thc. Agitated in ED, required IM medications (Haldol and Ativan) both 7/21 and 7/22. Pt lives with Mom and wanted to put a restraining order against Mom, though both mom and pt are on the rental lease.

Pt arrived cooperative but labile, cycling between irritation, crying spells, and fatigue. Signed CV. VSS. States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away." Pt confirms diagnoses and reports he had a VP shunt revision 20 years ago. ROI signed for mom (Terry) in order to get medication information/ appointment information, as pt did not know all of his meds or doses, stating "they expire so I don't take them and I didn't fill them. They are always expiring". Mom called for medications list. She states he is noncompliant and has been off meds for at least "a couple months." Home medications include buspirone 10mg bid, amlodipine 10mg, metoprolol (dose unknown), hydrochlorothiazide 25mg, omeprazole 20mg, fluoxetine 40mg, and lorazepam 1mg, divalproex 500mg, 0.5 tab PRN for acute anxiety or agitation and 1 tab hs. Daily 1.5 to 2 packs cigarette smoker. Daily 0.5 oz cannabis user. Twice weekly drinker (6 beers/ 2 shots). Pt has upcoming appt with new therapist on 8/8. Perseverating on mom being the cause/ reason he is here. Perseverating on appointment that he thinks he has on Friday, but mom reports his appt isn't until 8/8. Pt states "There she goes lying again. This is all her fault."

15-minute checks, low suicide precautions, AA and seizure precautions. Telephone orders from Dr Marks: Continue medications except metoprolol, decrease PRN lorazepam to 0.5mg bid. Start nicotine patch and gum PRN.

Chief Complaint (In patient's own words)

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Discharge Summary

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/29/2025 10:24:14AM

Course of Treatment

Reason for Admission:

Patient was admitted to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. The patient reports frustration stemming from unresolved family disputes, notably concerning his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate specific reasons.

Clinical Course and Treatment:

Upon arrival, the patient presented as anxious, irritable, with a labile mood. His Depakote dosage was increased to 1000 mg, which resulted in a good therapeutic effect. His mood gradually improved over the course of hospitalization. A Depakote level was ordered, but results were unavailable at the time of discharge. The patient demonstrated good compliance with treatment recommendations and reported improvement in symptoms of mood and perceived safety.

Throughout hospitalization, the patient did not exhibit any unsafe behaviors, including aggression, suicidal ideation (SI), or self-harm (SIB). There were no reports of new medical issues or allergies, and he denied any worsening of pre-existing medical conditions. His medical status remained stable during his stay.

Assessment and Disposition:

The clinical presentation and progress indicated that the patient had reached maximum benefit from the inpatient program. Given his stability and lack of imminent danger to self or others, a decision was made to discharge him. The patient continues to deny SI, SIB, or homicidal ideation.

Plan and Recommendations:

Continue adherence to prescribed medication regimen, including Depakote. Follow up with psychiatrist, therapist, and primary care provider for ongoing stabilization and treatment.

Emphasized the importance of regular outpatient appointments to prevent relapse and decompensation of both medical and mental health conditions.

Patient was educated on recognizing early warning signs of mood destabilization and instructed to seek immediate help if symptoms worsen.

Summary:

The patient showed significant clinical improvement, with stabilization of mood and overall functioning. He is deemed safe for community reintegration with appropriate outpatient support and medication management.

Discharge Status:

Stable, improved, not an imminent risk to self or others.

Functional/ADL/Type

Check one of the following

None or less than two antipsychotic medication prescribed at discharge

Physical/Baseline Functional Status

Discharge Summary

Flaherty, Dallas G

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DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/29/2025 10:24:14AM**

Physical condition is stable	Yes
Ambulation	Normal
Activities of Daily Living (Grooming and Hygeine)	Able to Perform
Social functioning within normal limits/baseline	Yes
Condition at the Time of Discharge	

Include patient's strengths and risk factors, adjustment, ability to function upon discharge, including assesement of suicidal/homicidal ideation.

Discharge Type	Routine
Status on day of discharge	Improved mood
Anticipated Problems after discharge and suggested means for intervention	encouraged med adherence

MSE

Appearance	Appropriate
Behavior	Cooperative
Eye Contact	Appropriate
Speech	Appropriate
Orientation	Time and Place
Attention	Fair
Motor	Appropriate
Thought Process	Appropriate
Thought Content	Denies
Mood	Euthymic
Affect	Congruent

Discharge Summary

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Insight	Fair
How Tested/Assessed	Understanding severity of illness
Judgement	Fair
How Tested/Assessed	Able to anticipate outcomes
Recent Memory	Not Impaired
Direct Questioning	Name
Remote Memory	Not Impaired
Direct Questioning	Name of childhood town

Therapeutic Services

Columbia-Suicide Severity Rating Scale-Discharge

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had thoughts about killing yourself? **No**

If YES to 2, ask questions 3, 4, 5, and 6. If NO to 2, go directly to question 6

6. Have you done anything, started to do anything, or prepared to do anything to end your life? **No**

Examples: Collected pills, obtained a gun, gave away valuables, wrote a will or suicide note, took out pills but didn't swallow any, held a gun but changed your mind or it was grabbed from your hand, went to the roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Risk Level **Low Suicide Risk**

Violence or Harm to others

1. Are you having any thoughts of harming others? **No**

Outcome/justification for discharge **improved mood**

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Protective Factors: (Identify protective factors offsetting/mitigating identified risks)

"Absence of plan, intent or means" and Engages in formulation of treatment plan

Therapeutic Services

Aftercare Plans (Document the following)

Patient is discharged to **Home**

Document the address and phone number to where patient is discharging to

Address **na**

Phone Number **na**

Transportation from Hospital **Cab/Uber/etc**

Post-Discharge Goal(s) (in patients' own words) **' to stay safe'**

Outpatient Tobacco Cessation Counseling Treatment Provided **Refused**

Follow-Up Care/Referrals

Row	Resource/Facility	Provider Name	Appointment Date/Time	Address	Contact Number/Fax/Email	Reason
1	Cape Behavioral Health Center (North East)	*referral initiated*	tbd	340 Gifford St Falmouth, MA 02540		Therapy
2	Cape Behavioral Health Center (North East)	Alanya Bouchard	8/1 @ 2pm TELEHEALTH	340 Gifford St Falmouth, MA 02540		Psychiatry

Nursing

Test and Procedures

Discharge Summary

Flaherty, Dallas G

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Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/29/2025 10:24:14AM**

Laboratory Results **Copies provided**

X-Ray Results **N/A**

EKG Results **N/A**

Other Results? **No**

Results Pending at Discharge **Yes**

Explain **VPA**

Patient/Family Instructions completed

Medications **Education**

Diet **N/A**

Activity **No Restrictions**

Summary

Ability to return to work or school **does not work**

Summary

None

Is it appropriate to bill for this
documentation? **Yes**

99238 - Discharge < 30 Minutes **Yes**

I do hereby attest that this information is true, accurate and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Provider Signature **e-Signed by Okorafor, Ayodeji E, DNP at 07/29/2025 10:37**

General Note

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/29/2025 2:03:36PM**

Note

Patient is alert and verbally responsive. Denies having any pain or discomfort. No SI/SH noticed this shift. Patient verbalized of feeling great and ready to go home. RN educated patient about his medication and future appointments. Patient verbalized of understanding and will follow through. Patient went home with his belongings and used a lyft to go home.

Staff Signature

e-Signed by Nalunkuma, Faridah, RN at 07/29/2025 14:07

History and Physical Exam

Flaherty, Dallas G

MR#: 109931
Hosp #: 2501689
DOB: 11/03/1994
Admit Date: 07/22/2025
Observation Date/Time: 7/23/2025 9:13:15AM

Assessment

Date of Service: 7/23/2025 00:00

Chief Complaint (In patient's own words) States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

History of Present Illness 30 y/o male with hx of Bipolar, ASD, GAD, nicotine dependence, Essential HTN, Epilepsy, GERD is admitted from Boston medical hospital due to threatening statements. He denies any medical complaints.

Assessment

Allergy	Type	Reaction	Severity	Date	Code	System
Comments						
metoclopramide	-Drug Allergy	-----	-----	----	----	d00298-----MULTUM
BRUGID-						
NO FOOD ALLERGIES	Drug Allergy					

Medications

Medications

Medications

History and Physical Exam

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/23/2025 9:13:15AM

Home Meds:

Description	Brand	Dose	Route	Frequency	PRN
Indication	-----	-----	-----	-----	---
Although a patient record was found, no medications were available.					
amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY	
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
divalproex sodium Oral 500 mg ERT Epilepsy, unspecified, not (Once-Daily)	Depakote ER	500 mg	PO	DAILY	
intractable, without status					
epilepticus					
Notes:Once-Daily FLUoxetine 40 mg CAP Bipolar disorder, current	PROzac	40 mg	PO	DAILY	
episode manic without					
psychotic features, severe					
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY	
hypertension					
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P
disorder					
Notes:SALAD omeprazole Oral 20 mg DRC Notes:SALAD	PriLOSEC	20 mg	PO	DAILY	GERD

Admission Meds:

Description	Brand	Dose	Route	Frequency	PRN
Indication	-----	-----	-----	-----	---
amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY	
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
divalproex sodium Oral 500 mg ERT Epilepsy, unspecified, not (Once-Daily)	Depakote ER	500 mg	PO	DAILY	
intractable, without status					

History and Physical Exam

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epilepticus

Notes:Once-Daily

FLUoxetine 40 mg CAP

Bipolar disorder, current

PROzac

40 mg

PO

DAILY

episode manic without

psychotic features, severe

hydroCHLORothiazide Oral 25 mg TAB

Essential (primary)

Oretic

25 mg

PO

DAILY

hypertension

LORazepam Oral 0.5 mg TAB

Generalized anxiety

Ativan

0.5 mg

PO

BID

P

disorder

Notes:SALAD

omeprazole Oral 20 mg DRC

Notes:SALAD

PriLOSEC

20 mg

PO

DAILY

GERD

Past, Family, and Social History (PFSH)

Surgical History

VP shunt/ VP revision 20 years ago

Past Medical History

Bipolar, Seizures, Suicide attempts and "Bipolar disorder, current episode manic without psychotic features, severe, Autistic disorder, Generalized anxiety disorder, Personal history of traumatic brain injury, Alcohol abuse, uncomplicated, Attention-deficit hyperactivity disorder, unspecified type, Cannabis abuse, uncomplicated, Epilepsy, unspecified, not intractable, without status epilepticus, Essential (primary) hypertension, Nicotine dependence, unspecified, uncomplicated, Pervasive developmental disorder, unspecified, Presence of other specified devices"

Family History

Cardiovascular Disease, Psychiatric Illness, Cancer/Tumors and Diabetes

Martial Status

Engaged

Living Situation

Family/Sig. Other

Have you used tobacco within previous 30 days?

Yes

Choose one

Greater than ¼ pack / day or daily use of smokeless tobacco, pipe, cigars

History and Physical Exam

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Social History

Row	Used in last 30 Days	Age of 1st Use	Current Amount	Current Frequency	Current Duration	Date of Last Use/Amt	Route of Usage (IV, Smoking, etc.)
1	Nicotine	18	1.5 to 2 packs daily	Daily	12 years	Prior to ED Admittance	Inhalation
2	Cannabis	18	0.5 ounce daily	Daily	12 years	Prior to ED Admittance	Inhalation
3	ETOH	Unknown	Twice weekly: 6 beers/ 2 shots	Twice weekly	Unknown	Unknown	PO

Past, Family, and Social History (PFSH)

History of STD

No

Exam

Review of Systems

Constitutional

No Complaints

Eyes

No Complaints

ENMT

No Complaints

Cardiovascular

No Complaints

Respiratory

No Complaints

Gastrointestinal

No Complaints

Genitourinary

No Complaints

Musculoskeletal

Pain

Musculoskeletal Comments

Back pain

Integumentary/Breast

No Complaints

History and Physical Exam

Flaherty, Dallas G

MR#: **109931**

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Admit Date: **07/22/2025**

Observation Date/Time: **7/23/2025 9:13:15AM**

Neurological **No Complaints**

Psychiatric **No Complaints**

Endocrine **No Complaints**

Hematologic/Lymphatic **No Complaints**

Allergic/Immunologic **No Complaints**

Vital Signs

Systolic Blood Pressure **131 mmHg**

Diastolic Blood Pressure **91 mmHg**

Blood Pressure **131 / 91**

Heart Rate **102 beats/min**

Temperature **98 °F**

Respirations **18 brth/min**

Height **70 in**

Weight **201 lb**

SPO2 **96 %**

Physical Exam

General Apperance **Cooperative**

Orientation **Person, Time and Situation**

Hallucination **Denies**

HEENMT **Abnormal**

Wears glasses

Neck **WNL: Supple. No lymphadenopathy, thyromegaly, or masses. No carotid bruit. No JVD.**

History and Physical Exam

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Respiratory	WNL: clear to auscultation bilaterally. No wheezing or crackles heard. No deformity or tenderness noted.
Cardiovascular	WNL: Regular rate and rhythm. No murmur, gallop, or rub noted. No S3 or S4 heard. PMI non displaced.
Chest/(Breasts)	WNL: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.
Gastrointestinal (Abdomen)	WNL: soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.
Genitourinary (Male/Female)	WNL: no complaints or concerns
Lymphatic	WNL (without tenderness)
Musculoskeletal	Abnormal surgical scar to R head, Hx of head surgery due to head bleed.
Skin	WNL: Skin is warm and dry. No rashes or lesions noted
Psychiatric	Abnormal admitted for psychiatry eval and treatment.
Neurological Exam	
(CN I) Olfactory	Normal
How Tested	Identification of cinnamon sample
(CN II) Optic Nerve	Normal
How Tested	Distinguishes number of fingers in central field
(CN III) Oculomotor	Normal
How Tested	Able to move eyes upward and outward
(CN IV) Trochlear	WNL - EOM's intact without lid lag or nystagmus
How Tested	Able to move eyes medially (downward at nose)
(CN V) Trigeminal	Normal
How Tested	Able to clench teeth and feels touch on front of head

History and Physical Exam

Flaherty, Dallas G

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(CN VI) Abducens	Normal
How Tested	Able to move eyes to either side
(CN VII) Facial	Normal
How Tested	Able to lift both eyebrows
CN VIII – Vestibulocochlear	Normal
How Tested	Able to hear finger rubs in both ears
(CN IX) Glossopharyngeal	Normal
How Tested	Swallow intact
(CN X) Vagus	Normal
How Tested	Able to lift palate when saying “Ah”
(CN XI) Accessory	Normal
How Tested	Able to shrug shoulders equally
CN XII – Hypoglossal	Normal
How Tested	Able to stick tongue out straight without tremors or fasciculation
Motor/Sensory Functions	
Gait and station	Normal
Romberg test	not performed
Muscle tone	Normal
No signs of limb weakness, atrophy and fasciculation	Normal
Deep tendon reflexes are 2+ and symmetrical	not performed
Plantar responses are down going	Normal
Finger- to- nose	Normal

History and Physical Exam

Flaherty, Dallas G

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Heel-to-shin **Normal**

Sensory - **Normal**
pain/touch/vibration/position

Lab Results/Consultation/Record **as cleared by ED**
Review

Medical Diagnosis

Exam

Diagnosis Code	System	Class	Type	Priority	Date
Personal history of traumatic Z87.820	ICD-10	--Medical	---Working	-----	----
brain injury					

ATTESTATION

I,

Darko, Sylvia

have reviewed and updated the above diagnoses as appropriate

Yes

INITIAL CERTIFICATION

I certify that inpatient psychiatric **Yes**
hospital services furnished are
necessary for either: (1) treatment
which could reasonably be expected
to improve the patient's condition, or
(2) diagnostic study.

I also certify that the patient **Yes**
continues to need, on a daily basis,
active treatment furnished directly by
or requiring the supervision of
inpatient psychiatric facility
personnel.

Treatment Plan and Recommendations

History and Physical Exam

Flaherty, Dallas G

MR#: **109931**

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DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/23/2025 9:13:15AM**

Problem: Altered Cardiac Output - Hypertension/Hypotension (Medication regimen, Verbal report of history of Hypertension)

- Dallas will be able to identify signs and symptoms of increased BP by time of discharge (Goal)
- Dallas will take medications as ordered (Objective)
- Nursing staff to administer medication and treatment(s) as ordered per MD (Intervention)

Problem: Nicotine Replacement/Smoking Cessation (Use of nicotine patch or gum per facility protocol, History of excessive nicotine use)

- Dallas will be compliant with nicotine replacement (Goal)
- Dallas will practice an identified coping skill to manage/reduce smoking urges (Objective)

- Nursing will administer nicotine replacement protocol as ordered per MD (Intervention)

Problem: Seizures (Alteration In Neurologic Homeostasis) (History of seizures. Last seizure in 2002 per pt. Pt noncompliant with medications, including seizure prevention meds.)

- Dallas will follow up with primary care physician for continued issues related to seizures (Goal)
- Dallas will exhibit an understanding of their seizure disorder prior to discharge (Objective)
- Staff will monitor patient as ordered by physician (Intervention)

Treatment Plan and Recommendations

History and Physical Exam

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/23/2025 9:13:15AM

Medication Orders:

Description	Brand	Dose	Route	Frequency	PRN
amlodipine Oral 10 mg TAB Essential (primary) -----	Norvasc	10 mg	PO	DAILY	---
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
divalproex sodium Oral 500 mg ERT Epilepsy, unspecified, not (Once-Daily) intractable, without status	Depakote ER	500 mg	PO	DAILY	
epilepticus Notes: Once-Daily					
FLUoxetine Oral 20 mg CAP Bipolar disorder, current	PROzac	40 mg	PO	DAILY	
episode manic without					
psychotic features, severe					
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY	
hypertension					
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P
disorder					
Notes: SALAD					
nicotine Oral Transmucosal 2 mg Smoking Cessation GUM	Nicorette	4 mg	OM	Q2H	P
nicotine 21 mg/24 hr Transdermal Nicotine Dependence TERF	*nicotine patch 21mg	1	TD	DAILY	P
omeprazole Oral 20 mg DRC Notes: SALAD	PriLOSEC	20 mg	PO	DAILY	GERD

Ancillary Orders:

Description	Frequency	PRN
-----	-----	---
-Adult Admit to: Conditional Voluntary Adult		
-COMPREHENSIVE METABOLIC PANEL	IN AM-UC	
-CBC (INCLUDES DIFF/PLT)	IN AM-UC	
-Assault/Aggression Precautions		
-Seizure Precautions		
- LOW Suicide Precaution		
-Certification of Admission		

Notes: I certify that this patient's inpatient psychiatric hospital admission is medically necessary for treatment which can reasonably be expected to improve the patient's condition and/ or diagnostic study

History and Physical Exam

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-Community Meeting
-Therapeutic Recreation/Activity Therapy
-Psycho-educational Groups
-*Regular Diet
-History and Physical Exam
-Weight on admission and Weekly
-Vital Signs

WEEKLY
VS_BID_Ad
ult
ObsPre

-Review Precaution and Observation Orders

Notes:Nurse to review and confirm patient's precautions and observations and document in nursing notes

-Every 15 Minutes Observation

-Medical Diagnosis:

-Psychiatric Diagnosis:

-Psychotherapy Process Groups

-Dietary Consult: Consult - Nutrition screen score of 3 or More

Recommendations for Treatment
and/or Follow-up

30 y/o male with hx of Bipolar, ASD, GAD, nicotine dependence, Essential HTN, Epilepsy, GERD is admitted from Boston medical hospital due to threatening statements. He denies any medical complaints.

**ED lab work/screening
as cleared by ED**

Assessment

Bipolar, ASD, GAD, nicotine dependence, Essential HTN, Epilepsy, GERD

Plan

Chart reviewed. Patient is medically stable as cleared by ED

Resume home meds

Monitor BP

Seizure precautions

Monitor for safety

Is it appropriate to bill for this
documentation?

Yes

99223 - Initial Hospital Care - High
Complexity

Yes

I do hereby attest that this information is true, accurate and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Signatures

Staff Signature

e-Signed by Darko, Sylvia at 07/23/2025 10:47

Inpatient Nursing Admission Assessment

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/22/2025 7:25:27PM

Initial

Date/Time

7/22/2025 19:25

Chief Complaint (patient's own words)

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Reason for Admission

Dallas is a 30 y.o. male presenting to WBHH from Good Samaritan. Pt was sectioned by police, for HI (stating he was going to kill parents/ hit father) after an argument with his parents. PPHx of bipolar d/o, ASD, GAD, PDD nos, and ADHD. PMH of seizure d/o, TBI (brain bleed/ lack of oxygen at birth) and HTN. Allergy to Reglan. In-house urine tox positive for benzos and thc. Agitated in ED, required IM medications (Haldol and Ativan) both 7/21 and 7/22. Pt lives with Mom and wanted to put a restraining order against Mom, though both mom and pt are on the rental lease.

Pt arrived cooperative but labile, cycling between irritation, crying spells, and fatigue. Signed CV. VSS. States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away." Pt confirms diagnoses and reports he had a VP shunt revision 20 years ago. ROI signed for mom (Terry) in order to get medication information/ appointment information, as pt did not know all of his meds or doses, stating "they expire so I don't take them and I didn't fill them. They are always expiring". Mom called for medications list. She states he is noncompliant and has been off meds for at least "a couple months." Home medications include buspirone 10mg bid, amlodipine 10mg, metoprolol (dose unknown), hydrochlorothiazide 25mg, omeprazole 20mg, fluoxetine 40mg, and lorazepam 1mg, divalproex 500mg, 0.5 tab PRN for acute anxiety or agitation and 1 tab hs. Daily 1.5 to 2 packs cigarette smoker. Daily 0.5 oz cannabis user. Twice weekly drinker (6 beers/ 2 shots). Pt has upcoming appt with new therapist on 8/8. Perseverating on mom being the cause/ reason he is here. Perseverating on appointment that he thinks he has on Friday, but mom reports his appt isn't until 8/8. Pt states "There she goes lying again. This is all her fault."

15-minute checks, low suicide precautions, AA and seizure precautions. Telephone orders from Dr Marks: Continue medications except metoprolol, decrease PRN lorazepam to 0.5mg bid. Start nicotine patch and gum PRN.

Preferred Language

English

Preferred Name

Dallas

Preferred Pronoun

He/Him/His

Sexual Orientation

What was your assigned sex at birth?

Male

Inpatient Nursing Admission Assessment

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How do you currently describe yourself?

Male

Which of the following best represents how you think of your sexual orientation?

Sexual or Heterosexual

Initial

Learning Assessment

Patient prefers

Audio and Visual

Cultural or Religious Preferences

"Protestant I think"

Motivation to learn (rate)

1

Emotional Barriers to Learning

Labile mood and Abuse

Physical/Cognitive Limitations

Learning disability

Communication Barriers

None

Initial

Allergy Comments	Type	Reaction	Severity	Date	Code	System
metoclopramide	Drug Allergy	-----	-----	----	----	d00298-----MULTUM
NO FOOD ALLERGIES	Drug Allergy					

General Appearance

Disheveled. Sunburnt and picking at skin on scalp. Appears older than stated age.

Tobacco Use

Tobacco Use

Have you used tobacco within previous 30 days

Yes

Choose one

Greater than ¼ pack / day or daily use of smokeless tobacco, pipe, cigars

Tobacco Cessation Counseling Provided

Refused

Inpatient Nursing Admission Assessment

Flaherty, Dallas G

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Admit Date: **07/22/2025**

Observation Date/Time: **7/22/2025 7:25:27PM**

FDA Approved Cessation Medication offered **Refused**

Tobacco Cessation Provider Provided **Refused**

The Alcohol Use Disorders Identification Test (AUDIT-C)

How often do you have a drink containing alcohol? **2 to 3 times a week**

How many drinks containing alcohol do you have on a typical day when you are drinking? **7, 8, or 9**

How often do you have six or more drinks if female, or 8 or more drinks if male, on one occasion? **Weekly**

Audit-C Score **9**

Score is **5 or Greater**

****SCORES OF 5 + REQUIRES THE FOLLOWING 7 QUESTIONS TO BE COMPLETED**

How often during the last year have you found that you weren't able to stop drinking once you started? **Never**

How often during the last year have you failed to do what was normally expected from you because of drinking? **Never**

How often during the last year have you been unable to remember what happened the night before because you had been drinking? **Never**

How often during the last year have you needed an alcoholic drink first thing in the morning to get yourself going after a night of heavy drinking? **Never**

How often during the last year have you had a feeling of guilt or remorse after drinking? **Never**

Have you or someone else been injured as a result of your drinking? **No**

Has a relative, friend, doctor, or another health professional expressed concern about your drinking or suggested you cut down? **Yes, during the last year**

AUDIT Score: **4**

Inpatient Nursing Admission Assessment

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Observation Date/Time: **7/22/2025 7:25:27PM**

AUDIT-C and AUDIT Total Score **13**

Scores between 0 and 7 - Lower Risk - No Intervention Required

Scores between 8 and 15 - Increasing Risk - Brief Advice

Scores between 16 and 19 - Higher Risk - Brief Advice and/or extended BA

Scores of 20+ - Possible Dependence - Referral to Services

Patient **Refused**

Physical Status

Physical Status

Systolic Blood Pressure **131 mmHg**

Diastolic Blood Pressure **91 mmHg**

Blood Pressure **131 / 91**

Heart Rate **102 beats/min**

Temperature **98 °F**

Respirations **18 brth/min**

Height **70 in**

Weight **201 lb**

SPO2 **96 % Room Air**

Brief Psychiatric Rating Scale

Somatic Concern **(3) Moderate**

Guilty Feelings **(0) Not Present**

Emotional Withdrawal **(2) Mild**

Inpatient Nursing Admission Assessment

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Conceptual Disorganization	(3) Moderate
Tension	(3) Moderate
Mannerisms and Posturing	(3) Moderate
Grandiosity	(2) Mild
Depressive Mood	(2) Mild
Hostility	(3) Moderate
Suspiciousness	(2) Mild
Hallucinatory Behavior	(0) Not Present
Motor Retardation	(0) Not Present
Blunted Affect	(0) Not Present
Uncooperativeness	(2) Mild
Unusual Thought Content	(0) Not Present
Anxiety	(2) Mild
Excitement	(3) Moderate
Disorientation	(2) Mild
Brief Psychiatric Rating Scale Total	32

Sleep/Vision/Hearing/Speech

Sleep, Vision, Hearing, Speech

Average hours of sleep per day **5 to 6**

Hours

Sleep - Check all that apply **Insomnia, Early Awakening, Night Time Awakening and Difficulty Falling Asleep**

Inpatient Nursing Admission Assessment

Flaherty, Dallas G

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Vision - Check all that apply

Problems Denied

Hearing - Check all that apply

Problems Denied

Immunization History/Flu Screening

Immunization History

Patient attends a school for which enrollment is dependent upon current immunizations

No

Immunizations believed to be current

No

Have you had a Covid Vaccine?

No

Is it Flu season October-March?

No

Check all that apply: (1 – 4 Permanent Contra-Indications for taking the influenza vaccine)

*Reminder to address form INFORMED CONSENT/DECLINATION FOR PATIENT INFLUENZA VACCINE with Patient

Nutritional

Nutritional Assessment

BMI **28.8**

Total Score **0**

Hydration Assessment

Approximate Daily Fluid Intake:
(other than ETOH) **3 – 6 glasses/day**

If 0 – 2 glasses/day is selected, Patient may be dehydrated CALL PHYSICIAN to assess hydration status

Physical

Skin/Skin Integrity

Inpatient Nursing Admission Assessment

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/22/2025 7:25:27PM**

Skin Check

Image

Per Patient Report:

Rashes, Burns and Scars

Please explain any reported skin integrity problems below

Sunburn to head, face, neck and extremities. Surgical scars to abd and head (VP/VP revision). Rash to right side/ underarm area.

Staff Witness

Bah, Abu Bakarr Bakarr

Physical

Pain

Pain

Do you have pain now?

Yes

Wong-Baker FACES

Current Pain Score

5

Type of Pain

Throbbing and Aching

Frequency

Continuous

Tolerable Pain Level

2

Duration

Chronic

Location: (specify area)

Upper Back and Lower Back

Aggravating factors

Sitting

Current pain relief regimens:

Somewhat effective

Effects of pain on Daily Activities

Somewhat Limiting

Inpatient Nursing Admission Assessment

Flaherty, Dallas G

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DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time:

Humpty Dumpty

Humpty Dumpty Fall Assessment

Total Humpty Dumpty score: **0**

Low Risk Humpty Dumpty score = 7-11

High Risk Humpty Dumpty score = 12 or above

Edmonson Psychiatric Fall Risk Assessment

Sleep Disturbance	12= Report of Sleep Disturbance by patient, family or staff
History of Falls	8=No history of falls
Mental Status	12=Agitation/Anxiety
Diagnosis	10=Bipolar/ Schizoaffective Disorder and 8=Substance abuse/Alcohol abuse
Elimination	8=Independent with control of bowel/bladder
Medication	10=No Medications
Ambulation/Balance	7= Independent/Steady gait/Immobile
Nutrition	0=No apparent abnormalities with appetite
Age	8 = Less than 50
Total Score	83

Strengths and Liabilities

Inventory of Strengths and Liabilities

Patient's Strengths	Stable Housing, Supportive Family/Friends and Hx of Psychiatric Stability or Sobriety
Patient's Limitations	Lack of Motivation, Unemployed and Perseverating on mom/ blame shifting
Patient & Family Educational Needs	Community Resources and Outpatient Programs

Inpatient Nursing Admission Assessment

Flaherty, Dallas G

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Admit Date: 07/22/2025

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Review Of Systems

Past Medical History per Intake	Bipolar, Seizures, Suicide attempts and "Bipolar disorder, current episode manic without psychotic features, severe, Autistic disorder, Generalized anxiety disorder, Personal history of traumatic brain injury, Alcohol abuse, uncomplicated, Attention-deficit hyperactivity disorder, unspecified type, Cannabis abuse, uncomplicated, Epilepsy, unspecified, not intractable, without status epilepticus, Essential (primary) hypertension, Nicotine dependence, unspecified, uncomplicated, Pervasive developmental disorder, unspecified, Presence of other specified devices"
Surgical History	VP shunt/ VP revision 20 years ago
Review of Systems (Check all that apply)	
Physical	Problems denied
Occupational (Check areas requiring assistance)	Problems denied
Swallowing	Problems denied
ENMT	No Complaints
Cardiovascular	No Complaints
Respiratory	No Complaints
Central Nervous System	Seizure disorder and TBI
Gastrointestinal	No Complaints
Genitourinary	No Complaints
Musculoskeletal	Pain
Musculoskeletal Comments	Back pain
Integumentary/Breast	No Complaints

Inpatient Nursing Admission Assessment

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Observation Date/Time: 7/22/2025 7:25:27PM

RN Narrative and Conclusions

Dallas is a 30 y.o. male presenting to WBHH from Good Samaritan. Pt was sectioned by police, for HI (stating he was going to kill parents/ hit father) after an argument with his parents. PPHx of bipolar d/o, ASD, GAD, PDD nos, and ADHD. PMH of seizure d/o, TBI (brain bleed/ lack of oxygen at birth) and HTN. Allergy to Reglan. In-house urine tox positive for benzos and thc. Agitated in ED, required IM medications (Haldol and Ativan) both 7/21 and 7/22. Pt lives with Mom and wanted to put a restraining order against Mom, though both mom and pt are on the rental lease.

Pt arrived cooperative but labile, cycling between irritation, crying spells, and fatigue. Signed CV. VSS. States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away." Pt confirms diagnoses and reports he had a VP shunt revision 20 years ago. ROI signed for mom (Terry) in order to get medication information/ appointment information, as pt did not know all of his meds or doses, stating "they expire so I don't take them and I didn't fill them. They are always expiring". Mom called for medications list. She states he is noncompliant and has been off meds for at least "a couple months." Home medications include buspirone 10mg bid, amlodipine 10mg, metoprolol (dose unknown), hydrochlorothiazide 25mg, omeprazole 20mg, fluoxetine 40mg, and lorazepam 1mg, divalproex 500mg, 0.5 tab PRN for acute anxiety or agitation and 1 tab hs. Daily 1.5 to 2 packs cigarette smoker. Daily 0.5 oz cannabis user. Twice weekly drinker (6 beers/ 2 shots). Pt has upcoming appt with new therapist on 8/8. Perseverating on mom being the cause/ reason he is here. Perseverating on appointment that he thinks he has on Friday, but mom reports his appt isn't until 8/8. Pt states "There she goes lying again. This is all her fault." Pt endorses 2 suicide attempts occurring around 10 years ago (via strangulation). Denies current SI/HI/AVH. No prior IPLOC hospitalizations.

15-minute checks, low suicide precautions, AA and seizure precautions. Telephone orders from Dr Marks: Continue medications except metoprolol, decrease PRN lorazepam to 0.5mg bid. Start nicotine patch and gum PRN.

Review Of Systems

Inpatient Nursing Admission Assessment

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/22/2025 7:25:27PM

Psychiatric

Problem - Danger To Others r/t Physical Aggression, Homicidal (HI statements towards parents, property destruction,) - Acute Care 07/22/2025

Goal - On a scale of 1-10 (where 1=manageable and 10=unmanageable) patient will report diminished Homicidal ideation and/or Danger to others and/or Physical aggression from a 9 to a 3 07/22/2025 07/30/2025

Objective - Patient will, prompted or independently, use a self-identified regulation skill to safely manage danger to others, homicidal ideation, and/or physical aggression. 07/22/2025 07/30/2025

Intervention - Conduct group (art, music, animal, movement, expressive) and monitor patient progress for the development of new leisure skills to decrease danger to others, homicidal ideation, and/or physical aggression TWICE A DAY 07/22/2025 Morrison, Edith

Intervention - Nursing will when Dallas identified with potential loss of control, ask Dallas what PRN medication is most helpful and administer medication according to doctor's order. TWICE A DAY 07/22/2025 Morrison, Edith

Intervention - Nursing will monitor Dallas as ordered by physician TWICE A DAY 07/22/2025 Morrison, Edith

Problem - Danger to Self (drinking -intoxication, punching objects) - Acute Care 07/22/2025

Objective - Patient will, prompted or independently, use a self-identified regulation skill to safely manage suicidal ideation and/or danger to self. 07/22/2025 07/30/2025

Intervention - Conduct group (art, music, animal, movement, expressive) and monitor patient progress for the development of new leisure skills to decrease danger to others, homicidal ideation, and/or physical aggression TWICE A DAY 07/22/2025 Morrison, Edith

Intervention - Nursing will monitor Dallas as ordered by physician As Needed 07/22/2025 Morrison, Edith

Medical

Problem - Altered Cardiac Output - Hypertension/Hypotension (Medication regimen, Verbal report of history of Hypertension) - Acute Care 07/22/2025

Nursing Signature

e-Signed by Avramidis, Angela at 07/22/2025 19:39

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time:

INPATIENT PSYCHIATRIC PROGRESS NOTE

Date of Service: 7/24/2025 00:00

Chief Complaint "in patient's own words" States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Current or Interval Psychiatric History patient continues to display emotional lability and accusatory speech. Despite expressing a desire for discharge and claiming symptom improvement, he remains visibly distressed about family issues. The clinical team observes that his affect is incongruent with his stated mood, and he exhibits some agitation. The patient's safety appears maintained at this time; however, his statements suggest ongoing stress related to family conflicts.

Plan & Recommendations:
Continue inpatient hospitalization for stabilization and safety monitoring
Medications will be maintained and adjusted as needed
The clinical team will arrange a follow-up outpatient appointment

Medication Side Effects No

Compliance with Treatment Yes

Suicidal Ideation No

Homicidal Ideation No

MENTAL STATUS EXAM

Appearance "Neat and Clean"

Behavior Impulsive

Orientation Person and Place

Concentration Inattentive

Speech Normal

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Insight	Poor
Memory	Fair
Judgement	Poor
Affect	Labile
Mood	Anxious
Hallucinations	Denies
Delusions	Patient currently denies
Significant Labs & Dates	none
Assessment	Unchanged
Rationale for Continued Inpatient Level of Care	stabilization

MEDICATION / ANCILLARY
ORDERS

Medication Orders:

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A
MR#: 109931
Hosp #: 2501689
DOB: 11/03/1994
Admit Date: 07/22/2025

amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY	
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
cloNIDine (Catapres) Oral 0.1 mg Elevated BP above 140/90.	Catapres	0.1 mg	PO	TID	P
Notes:Hold if HR<60 OR BP<90/60 or if symptomatic					
SALAD TAB					
divalproex sodium Oral 500 mg ERT Epilepsy,unspecified,not (Once-Daily)	Depakote	500 mg	PO	DAILY	
intractable,without status	ER				
epilepticus					
Notes:Once-Daily					
FLUoxetine Oral 20 mg CAP disorder,current	PROzac	40 mg	PO	DAILY	Bipolar episode
manic without					
psychotic features,severe					
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY	
hypertension					
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P
disorder					
Notes:SALAD					
nicotine Oral Transmucosal 2 mg Cessation	Nicorette	4 mg	OM	Q2H	P Smoking
GUM					
nicotine 21 mg/24 hr Transdermal Nicotine Dependence	*nicotine	1	TD	DAILY	P
TERF	patch 21mg	PATCH			
omeprazole Oral 20 mg DRC Notes:SALAD	PriLOSEC	20 mg	PO	DAILY	GERD
acetaminophen Oral 325 mg TAB Notes:NTE 3000 mg/day	*Tylenol	650 mg	PO	Q6H	P pain

Ancillary Orders:

Description

-Adult_Admit to: Conditional Voluntary Adult
-COMPREHENSIVE METABOLIC PANEL
-CBC (INCLUDES DIFF/PLT)
-Assault/Aggression Precautions

Frequency

PRN

IN AM-UC
IN AM-UC

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

- Seizure Precautions
- LOW Suicide Precaution
- Certification of Admission

Notes:I certify that this patient's inpatient psychiatric hospital admission is medically necessary for treatment which can reasonably be expected to improve the patient's condition and/ or diagnostic study

- Community Meeting
- Therapeutic Recreation/Activity Therapy
- Psycho-educational Groups
- *Regular Diet
- History and Physical Exam
- Weight on admission and Weekly
- Vital Signs

WEEKLY
VS_BID_Ad
ult
ObsPre

- Review Precaution and Observation Orders

Notes:Nurse to review and confirm patient's precautions and observations and document in nursing notes

- Every 15 Minutes Observation
- Medical Diagnosis:
- Psychiatric Diagnosis:
- Psychotherapy Process Groups

TREATMENT PLANNING

Problem: Danger To Others r/t Physical Aggression, Homicidal (HI statements towards parents, property destruction,)

OBSERVATION LEVELS AND PRECAUTIONS/CHANGE IN DIAGNOSIS

Current Level of Observation **Every 15 minutes**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

Change in Psychiatric
Diagnosis/Treatment **No**

Change in Medical
Diagnosis/Treatment **No**

Diagnosis	Class	Type	Priority	Date	Code
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Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

	Psychiatric	Provisiona	Primary Diagnosis
Bipolar disorder, current F31.13 ICD-10 episode manic without psychotic features, severe		1	
Autistic disorder F84.0 ICD-10	Psychiatric	Working	
Generalized anxiety disorder F41.1 ICD-10	Psychiatric	Working	
Personal history of traumatic Z87.820 ICD-10 brain injury	Medical	Working	
Alcohol abuse, uncomplicated F10.10 ICD-10		Working	
Attention-deficit F90.9 ICD-10 hyperactivity disorder, unspecified type		Working	
Cannabis abuse, uncomplicated F12.10 ICD-10		Working	
Epilepsy, unspecified, not G40.909 ICD-10 intractable, without status epilepticus		Working	
Essential (primary) I10 ICD-10 hypertension		Working	
Nicotine dependence, F17.200 ICD-10 unspecified, uncomplicated		Working	
Pervasive developmental F84.9 ICD-10 disorder, unspecified		Working	
Presence of other specified Z97.8 ICD-10 devices		Working	

Is it appropriate to bill for this documentation? **Yes**

99231 - Subsequent Hospital Low **Yes**

I do hereby attest that this information is true, accurate and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Provider Signature

e-Signed by Okorafor, Ayodeji E, DNP at 07/24/2025 12:52

Observation Date/Time:

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

INPATIENT PSYCHIATRIC PROGRESS NOTE

Date of Service: 7/25/2025 00:00

Chief Complaint "in patient's own words"

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Current or Interval Psychiatric History

Patient continues to demonstrate emotional lability and accusatory speech. Despite expressing a desire for discharge and reporting symptom improvement, he remains visibly distressed, particularly regarding ongoing family issues. The clinical team observes that his affect is incongruent with his stated mood, and he exhibits some agitation, including pacing the milieu.

The patient's safety appears to be maintained at this time. However, his statements indicate ongoing stress related to family conflicts. He was observed pacing the milieu, which coincided with increased blood pressure measured by medical staff; he is currently on amlodipine and has been started on PRN clonidine 0.1 mg for blood pressure management.

Medication adjustments include an increase of Depakote to 1000 mg daily. Labs have been ordered for tomorrow to monitor therapeutic levels and overall clinical status. Continued monitoring of mood, safety, and stress levels is warranted.

Medication Side Effects

No

Compliance with Treatment

Yes

Suicidal Ideation

No

Homicidal Ideation

No

MENTAL STATUS EXAM

Appearance

"Neat and Clean"

Behavior

Impulsive

Orientation

Person and Place

Concentration

Inattentive

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Speech	Pressured
Insight	Poor
Memory	Fair
Judgement	Poor
Affect	Labile
Mood	Euthymic
Hallucinations	Denies
Delusions	Patient currently denies
Significant Labs & Dates	none
Assessment	Unchanged
Rationale for Continued Inpatient Level of Care	stabilization

MEDICATION / ANCILLARY
ORDERS

Medication Orders:

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A
MR#: 109931
Hosp #: 2501689
DOB: 11/03/1994
Admit Date: 07/22/2025

amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY		
hypertension						
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID		
disorder						
cloNIDine (Catapres) Oral 0.1 mg Elevated BP above 140/90.	Catapres	0.1 mg	PO	TID		
Notes:Hold if HR<60 OR BP<90/60 or if symptomatic						
SALAD TAB						
divalproex sodium Oral 500 mg ERT Epilepsy,unspecified,not (Once-Daily)	Depakote	1,000	PO	DAILY		
intractable,without status	ER	mg				
epilepticus						
Notes:Once-Daily						
FLUoxetine Oral 20 mg CAP Anxiety,current episode	PROzac	20 mg	PO	DAILY		
without psychotic						manic
features,severe						
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY		
hypertension						
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P	
disorder						
Notes:SALAD						
nicotine Oral Transmucosal 2 mg Cessation	Nicorette	4 mg	OM	Q2H	P	Smoking
GUM						
nicotine 21 mg/24 hr Transdermal Nicotine Dependence	*nicotine	1	TD	DAILY	P	
TERF	patch 21mg	PATCH				
omeprazole Oral 20 mg DRC Notes:SALAD	PriLOSEC	20 mg	PO	DAILY		GERD
acetaminophen Oral 325 mg TAB Notes:NTE 3000 mg/day	*Tylenol	650 mg	PO	Q6H	P	pain

Ancillary Orders:

Description

-Adult_Admit to: Conditional Voluntary Adult
-COMPREHENSIVE METABOLIC PANEL
-CBC (INCLUDES DIFF/PLT)
-Assault/Aggression Precautions

Frequency

PRN

IN AM-UC

IN AM-UC

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

-Seizure Precautions

- LOW Suicide Precaution

-Certification of Admission

Notes:I certify that this patient's inpatient psychiatric hospital admission is medically necessary for treatment which can reasonably be expected to improve the patient's condition and/ or diagnostic study

-Community Meeting

-Therapeutic Recreation/Activity Therapy

-Psycho-educational Groups

-Psychotherapy Process Groups

-*Regular Diet

-History and Physical Exam

-Weight on admission and Weekly

-Vital Signs

WEEKLY

VS_BID_Ad

ult

ObsPre

-Review Precaution and Observation Orders

Notes:Nurse to review and confirm patient's precautions and observations and document in nursing notes

-Every 15 Minutes Observation

-Medical Diagnosis:

-Psychiatric Diagnosis:

-VALPROIC ACID

INAMUC

TREATMENT PLANNING

Problem: Danger To Others r/t Physical Aggression, Homicidal (HI statements towards parents, property destruction,)

OBSERVATION LEVELS AND PRECAUTIONS/CHANGE IN DIAGNOSIS

Current Level of Observation **Every 15 minutes**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

Change in Psychiatric
Diagnosis/Treatment **No**

Change in Medical
Diagnosis/Treatment **No**

Diagnosis	Class	Type	Priority	Date	Code
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Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

	Psychiatric	Provisiona	Primary Diagnosis
Bipolar disorder, current F31.13 ICD-10 episode manic without psychotic features, severe		1	
Autistic disorder F84.0 ICD-10	Psychiatric	Working	
Generalized anxiety disorder F41.1 ICD-10	Psychiatric	Working	
Personal history of traumatic Z87.820 ICD-10 brain injury	Medical	Working	
Alcohol abuse, uncomplicated F10.10 ICD-10		Working	
Attention-deficit F90.9 ICD-10 hyperactivity disorder, unspecified type		Working	
Cannabis abuse, uncomplicated F12.10 ICD-10		Working	
Epilepsy, unspecified, not G40.909 ICD-10 intractable, without status epilepticus		Working	
Essential (primary) I10 ICD-10 hypertension		Working	
Nicotine dependence, F17.200 ICD-10 unspecified, uncomplicated		Working	
Pervasive developmental F84.9 ICD-10 disorder, unspecified		Working	
Presence of other specified Z97.8 ICD-10 devices		Working	

Is it appropriate to bill for this documentation? **Yes**

99231 - Subsequent Hospital Low **Yes**

I do hereby attest that this information is true, accurate and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Provider Signature

e-Signed by Okorafor, Ayodeji E, DNP at 07/25/2025 11:48

Observation Date/Time:

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

INPATIENT PSYCHIATRIC PROGRESS NOTE

Date of Service: **7/26/2025 00:00**

Chief Complaint "in patient's own words" **States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."**

Current or Interval Psychiatric History **Patient is visible on the milieu, he is irritable mood continues to be labile, exit seeking, poorly follows staff redirections, agitated, encouraged medication adherence, tolerating current dose of Depakote with no reported side effect.**

Medication Side Effects **No**

Compliance with Treatment **Yes**

Suicidal Ideation **No**

Homicidal Ideation **No**

MENTAL STATUS EXAM

Appearance **"Neat and Clean"**

Behavior **Impulsive**

Orientation **Person and Place**

Concentration **Inattentive**

Speech **Pressured**

Insight **Poor**

Memory **Fair**

Judgement **Poor**

Affect **Labile**

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Mood	Euthymic
Hallucinations	Denies
Delusions	Patient currently denies
Significant Labs & Dates	none
Assessment	Unchanged
Rationale for Continued Inpatient Level of Care	stabilization

MEDICATION / ANCILLARY
ORDERS

Medication Orders:

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A
MR#: 109931
Hosp #: 2501689
DOB: 11/03/1994
Admit Date: 07/22/2025

amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY		
hypertension						
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID		
disorder						
cloNIDine (Catapres) Oral 0.1 mg Elevated BP above 140/90.	Catapres	0.1 mg	PO	TID		
Notes:Hold if HR<60 OR BP<90/60 or if symptomatic						
SALAD TAB						
divalproex sodium Oral 500 mg ERT Epilepsy,unspecified,not (Once-Daily)	Depakote	1,000	PO	DAILY		
intractable,without status	ER	mg				
epilepticus						
Notes:Once-Daily						
FLUoxetine Oral 20 mg CAP Anxiety,current episode	PROzac	20 mg	PO	DAILY		
without psychotic						manic
features,severe						
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY		
hypertension						
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P	
disorder						
Notes:SALAD						
nicotine Oral Transmucosal 2 mg Cessation	Nicorette	4 mg	OM	Q2H	P	Smoking
GUM						
nicotine 21 mg/24 hr Transdermal Nicotine Dependence	*nicotine	1	TD	DAILY	P	
TERF	patch 21mg	PATCH				
omeprazole Oral 20 mg DRC Notes:SALAD	PriLOSEC	20 mg	PO	DAILY		GERD
acetaminophen Oral 325 mg TAB Notes:NTE 3000 mg/day	*Tylenol	650 mg	PO	Q6H	P	pain

Ancillary Orders:

Description

Frequency

PRN

-Adult_Admit to: Conditional Voluntary Adult
-COMPREHENSIVE METABOLIC PANEL
-CBC (INCLUDES DIFF/PLT)
-Assault/Aggression Precautions

IN AM-UC
IN AM-UC

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

-Seizure Precautions

- LOW Suicide Precaution

-Certification of Admission

Notes:I certify that this patient's inpatient psychiatric hospital admission is medically necessary for treatment which can reasonably be expected to improve the patient's condition and/ or diagnostic study

-Community Meeting

-Therapeutic Recreation/Activity Therapy

-Psycho-educational Groups

-Psychotherapy Process Groups

-*Regular Diet

-History and Physical Exam

-Weight on admission and Weekly

-Vital Signs

WEEKLY

VS_BID_Ad

ult

ObsPre

-Review Precaution and Observation Orders

Notes:Nurse to review and confirm patient's precautions and observations and document in nursing notes

-Every 15 Minutes Observation

-Medical Diagnosis:

-Psychiatric Diagnosis:

-VALPROIC ACID

INAMUC

TREATMENT PLANNING

Problem: Danger To Others r/t Physical Aggression, Homicidal (HI statements towards parents, property destruction,)

OBSERVATION LEVELS AND PRECAUTIONS/CHANGE IN DIAGNOSIS

Current Level of Observation **Every 15 minutes**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

Change in Psychiatric
Diagnosis/Treatment **No**

Change in Medical
Diagnosis/Treatment **No**

Diagnosis	Class	Type	Priority	Date	Code
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Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

	Psychiatric	Provisiona	Primary Diagnosis
Bipolar disorder, current F31.13 ICD-10 episode manic without psychotic features, severe		1	
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Generalized anxiety disorder F41.1 ICD-10	Psychiatric	Working	
Personal history of traumatic Z87.820 ICD-10 brain injury	Medical	Working	
Alcohol abuse, uncomplicated F10.10 ICD-10		Working	
Attention-deficit F90.9 ICD-10 hyperactivity disorder, unspecified type		Working	
Cannabis abuse, uncomplicated F12.10 ICD-10		Working	
Epilepsy, unspecified, not G40.909 ICD-10 intractable, without status epilepticus		Working	
Essential (primary) I10 ICD-10 hypertension		Working	
Nicotine dependence, F17.200 ICD-10 unspecified, uncomplicated		Working	
Pervasive developmental F84.9 ICD-10 disorder, unspecified		Working	
Presence of other specified Z97.8 ICD-10 devices		Working	

Is it appropriate to bill for this documentation? **Yes**

99231 - Subsequent Hospital Low **Yes**

I do hereby attest that this information is true, accurate and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Provider Signature

e-Signed by Okorafor, Ayodeji E, DNP at 07/26/2025 12:09

Observation Date/Time:

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

INPATIENT PSYCHIATRIC PROGRESS NOTE

Date of Service: 7/27/2025 00:00

Chief Complaint "in patient's own words" States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Current or Interval Psychiatric History Patient is visible on the milieu, keeping to himself, no aggression, taking medications with good effect, attending groups. No current acute concerns reported.

Medication Side Effects No

Compliance with Treatment Yes

Suicidal Ideation No

Homicidal Ideation No

MENTAL STATUS EXAM

Appearance "Neat and Clean"

Behavior Impulsive

Orientation Person and Place

Concentration Inattentive

Speech Pressured

Insight Poor

Memory Fair

Judgement Poor

Affect Appropriate to Mood

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Mood	Euthymic
Hallucinations	Denies
Delusions	Patient currently denies
Significant Labs & Dates	none
Assessment	Unchanged
Rationale for Continued Inpatient Level of Care	stabilization

MEDICATION / ANCILLARY
ORDERS

Medication Orders:

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A
MR#: 109931
Hosp #: 2501689
DOB: 11/03/1994
Admit Date: 07/22/2025

amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY	
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
cloNIDine (Catapres) Oral 0.1 mg Elevated BP above 140/90.	Catapres	0.1 mg	PO	TID	P
Notes:Hold if HR<60 OR BP<90/60 or if symptomatic					
SALAD TAB					
divalproex sodium Oral 500 mg ERT Epilepsy,unspecified,not (Once-Daily)	Depakote	1,000	PO	DAILY	
intractable,without status	ER	mg			
epilepticus					
Notes:Once-Daily					
FLUoxetine Oral 20 mg CAP Anxiety,current episode	PROzac	20 mg	PO	DAILY	
					manic
without psychotic					
features,severe					
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY	
hypertension					
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P
disorder					
Notes:SALAD					
nicotine Oral Transmucosal 2 mg Cessation	Nicorette	4 mg	OM	Q2H	P Smoking
GUM					
nicotine 21 mg/24 hr Transdermal Nicotine Dependence	*nicotine	1	TD	DAILY	P
TERF	patch 21mg	PATCH			
omeprazole Oral 20 mg DRC Notes:SALAD	PriLOSEC	20 mg	PO	DAILY	GERD
acetaminophen Oral 325 mg TAB Notes:NTE 3000 mg/day	*Tylenol	650 mg	PO	Q6H	P pain

Ancillary Orders:

Description

-Adult_Admit to: Conditional Voluntary Adult
-COMPREHENSIVE METABOLIC PANEL
-CBC (INCLUDES DIFF/PLT)
-Assault/Aggression Precautions

Frequency

PRN

IN AM-UC
IN AM-UC

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

-Seizure Precautions

- LOW Suicide Precaution

-Certification of Admission

Notes:I certify that this patient's inpatient psychiatric hospital admission is medically necessary for treatment which can reasonably be expected to improve the patient's condition and/ or diagnostic study

-Community Meeting

-Therapeutic Recreation/Activity Therapy

-Psycho-educational Groups

-Psychotherapy Process Groups

-*Regular Diet

-History and Physical Exam

-Weight on admission and Weekly

-Vital Signs

WEEKLY

VS_BID_Ad

ult

ObsPre

-Review Precaution and Observation Orders

Notes:Nurse to review and confirm patient's precautions and observations and document in nursing notes

-Every 15 Minutes Observation

-Medical Diagnosis:

-Psychiatric Diagnosis:

-VALPROIC ACID

INAMUC

TREATMENT PLANNING

Problem: Danger To Others r/t Physical Aggression, Homicidal (HI statements towards parents, property destruction,)

OBSERVATION LEVELS AND PRECAUTIONS/CHANGE IN DIAGNOSIS

Current Level of Observation **Every 15 minutes**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

Change in Psychiatric
Diagnosis/Treatment **No**

Change in Medical
Diagnosis/Treatment **No**

Diagnosis	Class	Type	Priority	Date	Code
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Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

	Psychiatric	Provisiona	Primary Diagnosis
Bipolar disorder, current F31.13 ICD-10 episode manic without psychotic features, severe		1	
Autistic disorder F84.0 ICD-10	Psychiatric	Working	
Generalized anxiety disorder F41.1 ICD-10	Psychiatric	Working	
Personal history of traumatic Z87.820 ICD-10 brain injury	Medical	Working	
Alcohol abuse, uncomplicated F10.10 ICD-10		Working	
Attention-deficit F90.9 ICD-10 hyperactivity disorder, unspecified type		Working	
Cannabis abuse, uncomplicated F12.10 ICD-10		Working	
Epilepsy, unspecified, not G40.909 ICD-10 intractable, without status epilepticus		Working	
Essential (primary) I10 ICD-10 hypertension		Working	
Nicotine dependence, F17.200 ICD-10 unspecified, uncomplicated		Working	
Pervasive developmental F84.9 ICD-10 disorder, unspecified		Working	
Presence of other specified Z97.8 ICD-10 devices		Working	

Is it appropriate to bill for this documentation? **Yes**

99231 - Subsequent Hospital Low **Yes**

I do hereby attest that this information is true, accurate and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Provider Signature

e-Signed by Okorafor, Ayodeji E, DNP at 07/27/2025 11:58

Observation Date/Time:

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

INPATIENT PSYCHIATRIC PROGRESS NOTE

Date of Service: 7/28/2025 00:00

Chief Complaint "in patient's own words" States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Current or Interval Psychiatric History Patient is visible on the milieu, keeping to himself, no aggression, taking medications with good effect, attending groups. No current acute concerns reported. Discharge planning for Tuesday/ Wednesday pending VPA level

Medication Side Effects No

Compliance with Treatment Yes

Suicidal Ideation No

Homicidal Ideation No

MENTAL STATUS EXAM

Appearance "Neat and Clean"

Behavior Impulsive

Orientation Person and Place

Concentration Inattentive

Speech Normal

Insight Poor

Memory Fair

Judgement Poor

Affect Appropriate to Mood

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Mood	Euthymic
Hallucinations	Denies
Delusions	Patient currently denies
Significant Labs & Dates	none
Assessment	Unchanged
Rationale for Continued Inpatient Level of Care	d/c planning

**MEDICATION / ANCILLARY
ORDERS**

Medication Orders:

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A
MR#: 109931
Hosp #: 2501689
DOB: 11/03/1994
Admit Date: 07/22/2025

amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY	
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
cloNIDine (Catapres) Oral 0.1 mg Elevated BP above 140/90.	Catapres	0.1 mg	PO	TID	P
Notes:Hold if HR<60 OR BP<90/60 or if symptomatic					
SALAD TAB					
divalproex sodium Oral 500 mg ERT Epilepsy,unspecified,not (Once-Daily)	Depakote	1,000	PO	DAILY	
intractable,without status	ER	mg			
epilepticus					
Notes:Once-Daily					
FLUoxetine Oral 20 mg CAP Anxiety,current episode	PROzac	20 mg	PO	DAILY	
without psychotic					manic
features,severe					
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY	
hypertension					
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P
disorder					
Notes:SALAD					
nicotine Oral Transmucosal 2 mg Cessation	Nicorette	4 mg	OM	Q2H	P Smoking
GUM					
nicotine 21 mg/24 hr Transdermal Nicotine Dependence	*nicotine	1	TD	DAILY	P
TERF	patch 21mg	PATCH			
omeprazole Oral 20 mg DRC Notes:SALAD	PriLOSEC	20 mg	PO	DAILY	GERD
acetaminophen Oral 325 mg TAB Notes:NTE 3000 mg/day	*Tylenol	650 mg	PO	Q6H	P pain

Ancillary Orders:

Description

-Adult_Admit to: Conditional Voluntary Adult
-VALPROIC ACID
-COMPREHENSIVE METABOLIC PANEL
-CBC (INCLUDES DIFF/PLT)

Frequency

PRN

INAMUC
IN AM-UC
IN AM-UC

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

- Assault/Aggression Precautions
- Seizure Precautions
- LOW Suicide Precaution
- Certification of Admission

Notes:I certify that this patient's inpatient psychiatric hospital admission is medically necessary for treatment which can reasonably be expected to improve the patient's condition and/ or diagnostic study

- Community Meeting
- Therapeutic Recreation/Activity Therapy
- Psycho-educational Groups
- Psychotherapy Process Groups
- *Regular Diet
- History and Physical Exam
- Weight on admission and Weekly
- Vital Signs

WEEKLY
VS_BID_Ad
ult
ObsPre

- Review Precaution and Observation Orders

Notes:Nurse to review and confirm patient's precautions and observations and document in nursing notes

- Every 15 Minutes Observation
- Medical Diagnosis:
- Psychiatric Diagnosis:
- Patient may shave

Notes:supervised

- Patient may use for numbers

TREATMENT PLANNING

Problem: Danger To Others r/t Physical Aggression, Homicidal (HI statements towards parents, property destruction,)

OBSERVATION LEVELS AND PRECAUTIONS/CHANGE IN DIAGNOSIS

Current Level of Observation	Every 15 minutes
Current Precautions	LOW Suicide, Seizure, Assault/Aggression
Change in Psychiatric Diagnosis/Treatment	No
Change in Medical Diagnosis/Treatment	No

Inpatient Psychiatric Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Diagnosis	Class	Type	Priority	Date	Code
Bipolar disorder, current F31.13 ICD-10	Psychiatric	Provisional	Primary Diagnosis		
episode manic without psychotic features, severe		1			
Autistic disorder F84.0 ICD-10	Psychiatric	Working			
Generalized anxiety disorder F41.1 ICD-10	Psychiatric	Working			
Personal history of traumatic Z87.820 ICD-10	Medical	Working			
brain injury					
Alcohol abuse, uncomplicated F10.10 ICD-10		Working			
Attention-deficit F90.9 ICD-10		Working			
hyperactivity disorder, unspecified type					
Cannabis abuse, uncomplicated F12.10 ICD-10		Working			
Epilepsy, unspecified, not G40.909 ICD-10		Working			
intractable, without status epilepticus					
Essential (primary) I10 ICD-10		Working			
hypertension					
Nicotine dependence, F17.200 ICD-10		Working			
unspecified, uncomplicated					
Pervasive developmental F84.9 ICD-10		Working			
disorder, unspecified					
Presence of other specified Z97.8 ICD-10		Working			
devices					

Is it appropriate to bill for this documentation? **Yes**

99231 - Subsequent Hospital Low **Yes**

I do hereby attest that this information is true, accurate and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Provider Signature

e-Signed by Okorafor, Ayodeji E, DNP at 07/28/2025 12:11

Inpatient Psychiatric Progress Note

Westborough Behavioral Health Hospital

Flaherty, Dallas G

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Intake Assessment - Nursing Triage

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/22/2025 6:32:50PM**

Initial

Date/Time **7/22/2025 18:32**

Identification Section

Chief Complaint/Presenting Issues
(in patient's own words) **States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."**

Sex/Current Gender Identity or
Expression **Male**

If language/hearing services are needed, please arrange and complete disposition section under inpatient stay

Preferred Language **English**

Preferred Name **Dallas**

Race **White**

Preferred Pronoun **He/Him/His**

Referral Source **Good Samaritan**

Is person accompanied by anyone? **No**

MILITARY HISTORY

Is the patient active or former
military? **No**

Does patient have a designated
family caregiver to assist **Yes**

Patient caregiver is: **Relative**

Relationship to patient **Mother (Terry)**

Phone number **781-815-8001**

Is there a guardian signing in patient? **No**

Was a parent or other person notified
of patient admission? **Yes**

Pt's mother notified of admission

Intake Assessment - Nursing Triage

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time:

Vital Signs/History

Vital Signs

CONSULT PROVIDER IF:

Temp: > 100.1 Pulse: <50;>140 or irregular

Resp: <12 or >28 B/P: <90/50 or >180/110

Breathalyzer: > .350 Glucometer <50 or >350

SPO2 <90%

Temperature **98 °F**

Pulse **102 beats/min**

Respirations **18 brth/min**

Systolic Blood Pressure **131 mmHg**

Diastolic Blood Pressure **91 mmHg**

Blood Pressure **131 / 91**

O2 Sat **96 %**

Height **70 in**

Weight **201 lb**

BMI **28.8**

Current Physical Health Status (This section must be completed regardless of Disposition)

Current Medical Problems **"Bipolar disorder, current episode manic without psychotic features, severe, Autistic disorder, Generalized anxiety disorder, Personal history of traumatic brain injury, Attention-deficit hyperactivity disorder, unspecified type, Essential (primary) hypertension, Pervasive developmental disorder, unspecified, Presence of other specified devices" and Seizure d/o-last seizure was 2002 per pt.**

Enter/Review Allergies and Reactions

Intake Assessment - Nursing Triage

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/22/2025 6:32:50PM

Vital Signs/History

Allergy Comments	Type	Reaction	Severity	Date	Code	System
metoclopramide BRUGID--	--Drug Allergy	-----	-----	----	----	d00298-----MULTUM
NO FOOD ALLERGIES	Drug Allergy					

Communicable Disease Exposure - Do you currently have, or have you recently been exposed to, or have you ever had any of the following

Exposure-V2

Row	Currently Have	Recent Exposure (Last 30 days)	Had in past	Comments
Measles	No	No	No	
Mumps	No	No	No	
Rubella	No	No	No	
Chicken Pox	No	No	No	
Hepatitis	No	No	No	
HIV	No	No	No	

Vital Signs/History

Skin Irritation

Do you currently have any or have you had any recent skin irritations or rashes?

Yes

Red raised rash to right flank.

Have you been exposed to areas containing a bed bug infestation?

No

Current Medications

Yes

Home Medication Reconciliation to be done on the Orders Home Med List Tab

Vaccination Screen

Is assessment taking place during Flu Season (October 1 – March 31)

No

Intake Assessment - Nursing Triage

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/22/2025 6:32:50PM**

Current treatment of any infection or contagious disease such as	Denies
History of treatment of infection or contagious disease such as MRSA, VRE, C DIFF	Denies
Pain Assessment	
Indicate pain scale (0-No pain -10)	5
Cause of pain	Back pain
Location	Back
Open wounds	No
Level of Consciousness/Neurological Deficits	Oriented x 4, Confused/Disoriented, History of seizures, History of TBI and Move all extremities
Patient presenting for detox	No
Patient stable	Yes
Can patient be safely treated at this facility?	Yes
Any medical consultation recommendations?	No
Nurse Signature	e-Signed by Avramidis, Angela at 07/22/2025 18:44

Medical Consultation

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/24/2025 12:17:27PM**

Date of Service: **7/24/2025**

Reason for Consultation/Chief Complaint **Follow up, HTN**

History of Present Illness **30 y/o male with hx o0f HTN is seen for follow up and med management. BP is progressively elevated. Todays BP - 161/90. He states he is very anxious and wants to be discharge asap. Noted pacing on unit and tensed up. Denies any medical complaints.**

Medical History **HTN**

Surgical History **n/a**

Medical Consultation

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/24/2025 12:17:27PM

Description Indication	Brand	Dose	Route	Frequency	PRN
amlodipine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY	
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
clonidine (Catapres) Oral 0.1 mg Elevated BP above 140/90.	Catapres	0.1 mg	PO	TID	P
Notes:Hold if HR<60 OR BP<90/60 or if symptomatic					
SALAD TAB					
divalproex sodium Oral 500 mg ERT Epilepsy,unspecified,not (Once-Daily)	Depakote	500 mg	PO	DAILY	
intractable,without status	ER				
epilepticus					
Notes:Once-Daily					
FLUoxetine Oral 20 mg CAP Bipolar disorder,current	PROzac	40 mg	PO	DAILY	
episode manic without					
psychotic features,severe					
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY	
hypertension					
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P
disorder					
Notes:SALAD					
nicotine Oral Transmucosal 2 mg Smoking Cessation	Nicorette	4 mg	OM	Q2H	P
GUM					
nicotine 21 mg/24 hr Transdermal Nicotine Dependence	*nicotine	1	TD	DAILY	P
TERF	patch	PATCH			
omeprazole Oral 20 mg DRC	PriLOSEC	20 mg	PO	DAILY	GERD
Notes:SALAD					
acetaminophen Oral 325 mg TAB Notes:NTE 3000 mg/day	*Tylenol	650 mg	PO	Q6H	P pain

Allergy Comments	Type	Reaction	Severity	Date	Code	System
metoclopramide DRUGID	Drug Allergy				d00298	MULTUM
NO FOOD ALLERGIES	Drug Allergy					

Medical Consultation

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/24/2025 12:17:27PM**

Review of Systems (All systems reviewed were without pertinent findings except as noted)	PE unremarkable
Systolic Blood Pressure	161 mmHg
Diastolic Blood Pressure	90 mmHg
Blood Pressure	161 / 90
Pulse	100 %
Respirations	18 brth/min
Temperature	97.8 °F
O2	98 %
Vitals Time	n/a
Mask Offered	No
Vital Signs	
Heart Rate	100 beats/min
Height	70 in
Weight	202 lb
SPO2	98 %
Physical Exam	
General Apperance	Cooperative
Orientation	Person and Place
Hallucination	Denies
HEENMT	WNL: Normocephalic, atraumatic. Pupils equal, round and reactive to light symmetrically. EOM intact. Anicteric sclera. TM intact bilaterally. Nares clear. Oropharynx is clear. No erythema or exudates noted. No acute dental problems noted. Oral mucosa is moist.

Medical Consultation

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/24/2025 12:17:27PM

Neck	WNL: Supple. No lymphadenopathy, thyromegaly, or masses. No carotid bruit. No JVD.
Respiratory	WNL: clear to auscultation bilaterally. No wheezing or crackles heard. No deformity or tenderness noted.
Cardiovascular	WNL: Regular rate and rhythm. No murmur, gallop, or rub noted. No S3 or S4 heard. PMI non displaced.
Chest/(Breasts)	WNL: Clear to auscultation bilaterally. No wheezing or crackles heard. No deformity, or tenderness noted.
Gastrointestinal (Abdomen)	WNL: soft, normoactive bowel sounds, nondistended, nontender. No guarding or rebound. No organomegaly.
Genitourinary (Male/Female)	WNL: no complaints or concerns
Lymphatic	WNL (without tenderness)
Musculoskeletal	WNL (No clubbing, cyanosis, or edema. Pulses are present and equal bilaterally. No scoliosis, kyphosis, or abnormal lordosis. NO CVA tenderness)
Skin	WNL: Skin is warm and dry. No rashes or lesions noted
Psychiatric	Abnormal

admitted for psychiatry eval and treatment.

Medical Consultation

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/24/2025 12:17:27PM

Diagnosis Code	System	Class	Type	Priority	Date
Bipolar disorder, current F31.13 ICD-10		Psychiatric	Provisional	Primary Diagnosis	----
episode manic without psychotic features, severe			1		
Autistic disorder F84.0 ICD-10		Psychiatric	Working		
Generalized anxiety disorder F41.1 ICD-10		Psychiatric	Working		
Personal history of traumatic Z87.820 ICD-10		Medical	Working		
brain injury					
Alcohol abuse, uncomplicated F10.10 ICD-10			Working		
Attention-deficit F90.9 ICD-10			Working		
hyperactivity disorder, unspecified type					
Cannabis abuse, uncomplicated F12.10 ICD-10			Working		
Epilepsy, unspecified, not G40.909 ICD-10			Working		
intractable, without status epilepticus					
Essential (primary) I10 ICD-10			Working		
hypertension					
Nicotine dependence, F17.200 ICD-10			Working		
unspecified, uncomplicated					
Pervasive developmental F84.9 ICD-10			Working		
disorder, unspecified					
Presence of other specified Z97.8 ICD-10			Working		
devices					

Recommendations

A/P

A - HTN (BP elevated in the setting of anxiety)

Plan

Clonidine 0.1mg TID prn for BP above 140/90.

Monitor BP

Nursing to encouraged prn anxiety meds

Select one

Thank you for this consult. We will continue to follow this patient with you

Is it appropriate to bill for this
documentation?

Yes

Medical Consultation

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time:

Select the appropriate code below:

99232 - Subsequent Care (Medical Consultation) Moderate **Yes**

I do hereby attest that this information is true, accurate and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Attending Physician **as above**
Review/Comments

Staff Signature **e-Signed by Darko, Sylvia at 07/24/2025 12:25**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Progress Note

Observation Date/Time:

NURSING PROGRESS NOTE - INPATIENT

Date/Time **7/23/2025 00:05**

Shift **Night**

Reason for Admission **States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."**

History of Present Illness **Refer to Intake**

VITAL SIGNS

Systolic Blood Pressure **131 mmHg**

Diastolic Blood Pressure **91 mmHg**

Blood Pressure **131 / 91**

Heart Rate **102 beats/min**

Temperature **98 °F**

Respirations **18 brth/min**

Height **70 in**

Weight **201 lb**

SPO2 **96 %**

****RN: Orders must be renewed daily**

Special Interventions This Shift **None**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation **Every 15 minutes**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

PAIN

Pain today? **No**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Is the Patient already on Suicide Precautions? **Yes**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

6. Have you ever done anything, started to do anything, or prepared to do anything to end your life? **No**

Ex: Collected pills, obtained gun, gave away valuables, wrote will/suicide note, took out pills but didn't swallow, held gun but changed mind or it was grabbed from your hand, went to roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the patient's presentation? **No**

VULNERABILITY

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Is Vulnerability currently a part of the patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), i the previous 24
hours.

The items are then summed to
produce a total score.

The risk rating is determined based
on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	0

Record of aggression: During the
previous 24 hours, has the patient
behaved aggressively in any of the
following ways?

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Physical aggression against
OBJECTS **No**

Verbal aggression against OTHER
PEOPLE **No**

Physical Aggression against OTHER
PEOPLE **No**

DASA

If patient scored Moderate or High
Risk and not on Aggression/Assault
Precautions notify MD

CURRENT STATUS

L.O.C.	Alert and Asleep
Mobility	WNL
Activity	Normal and Asleep
Awareness	Aware of surroundings and Asleep
Behavior	In bed
Mood	Euthymic
Affect	WNL
Thought Process	Organized
Thought Content	WNL
Speech	Normal
Detox Patient	No
ADLs	Not taking care of ADLs

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Bathed/Showered	No
Appearance	Neat/Clean
Sleep	Slept throughout night

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Snack	Yes
Supplements	N/A
Elimination	Reports no problem and Asleep

REVIEW OF SYSTEMS

Review of Systems	WNL
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PATIENT EDUCATION

Topic	Medication Adherence
Level of Understanding	Partially understands Needs additional support for understanding
Nursing Shift Narrative	Isolative in room most of the shift. Out of room for bedtime medication. Appropriate and pleasant on interaction. No unsafe behavior reported or observed. Slept for more than 7hrs. Q15 minutes observation maintained.

****RN: Document to the treatment plan every shift!**

PATIENT EDUCATION

- Nursing will monitor Dallas as ordered by physician (Intervention)
- Nursing will monitor Dallas as ordered by physician (Intervention)
- Nursing staff to administer medication and treatment(s) as ordered per MD (Intervention)
- Nursing will administer nicotine replacement protocol as ordered per MD (Intervention)
- .Staff will monitor patient as ordered by physician (Intervention)

Nurse Signature	e-Signed by Azuike, Judith at 07/23/2025 06:23
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Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Progress Note

Observation Date/Time:

NURSING PROGRESS NOTE -
INPATIENT

Date/Time **7/23/2025 16:25**

Shift **Day**

Reason for Admission **States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure	120 mmHg
Diastolic Blood Pressure	74 mmHg
Blood Pressure	120 / 74
Heart Rate	103 beats/min
Temperature	97.8 °F
Respirations	16 brth/min
Height	70 in
Weight	202 lb
SPO2	98 %

****RN: Orders must be renewed daily**

Special Interventions This Shift **PRN Medications**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation **Every 15 minutes**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

PAIN

Pain today? **No**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Is the Patient already on Suicide Precautions? **No**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

6. Have you ever done anything, started to do anything, or prepared to do anything to end your life? **No**

Ex: Collected pills, obtained gun, gave away valuables, wrote will/suicide note, took out pills but didn't swallow, held gun but changed mind or it was grabbed from your hand, went to roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

The items are then summed to
produce a total score.

The risk rating is determined based
on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(1) - The patient is easily annoyed or angered and unable to tolerate the presence of others.
Impulsivity	(1) - The patient has been impulsive and unpredictable in his/her affect or behavior or was quick to (over-) react to real and imagined slights, insults, and disappointments.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	2

Record of aggression: During the
previous 24 hours, has the patient
behaved aggressively in any of the
following ways?

Physical aggression against
OBJECTS **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Verbal aggression against OTHER PEOPLE **Yes**

Physical Aggression against OTHER PEOPLE **No**

DASA

If patient scored Moderate or High Risk and not on Aggression/Assault Precautions notify MD

CURRENT STATUS

L.O.C.	Alert
Mobility	WNL
Activity	Normal
Awareness	Aware of surroundings
Behavior	Attending groups
Mood	Anxious and Irritable
Affect	Labile and Flat
Thought Process	Disorganized and Confused
Thought Content	Delusional
Speech	Normal
Detox Patient	No
ADLs	Taking care of ADLs
Bathed/Showered	Yes

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Appearance	Neat/Clean
Sleep	Napped during the day

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Meals	Eating off unit
Breakfast	100%
Lunch	100%
Dinner	100%
Snack	Yes
Supplements	N/A
Elimination	Reports no problem

REVIEW OF SYSTEMS

Review of Systems	WNL
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PATIENT EDUCATION

Topic	Safety
Level of Understanding	Partially understands Needs additional support for understanding
Nursing Shift Narrative	Patient presented with a flat affect, alert and verbally responsive. Denies having any pain or discomfort. Able to make his needs known. Noticed interacting well with selective peers. Positive for all his meds as ordered. Prn gum 2 mg . Joined the group meeting and coloring. Transited well at the cafe. He has good appetite. Staff educated patient how to use his coping skills. No issues reported this shift. will continue on 15 mins checks for safety.

**RN: Document to the treatment plan every shift!

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

PATIENT EDUCATION

- .Nursing will when Dallas identified with potential loss of control, ask Dallas what PRN medication is most helpful and administer medication according to doctor's order.

(Intervention)

- Nursing will monitor Dallas as ordered by physician (Intervention)
- Nursing will monitor Dallas as ordered by physician (Intervention)
- Nursing staff to administer medication and treatment(s) as ordered per MD (Intervention)
- Nursing will administer nicotine replacement protocol as ordered per MD (Intervention)
- .Staff will monitor patient as ordered by physician (Intervention)

Nurse Signature

e-Signed by Nalunkuma, Faridah, RN at 07/23/2025 16:34

Progress Note

Observation Date/Time:

NURSING PROGRESS NOTE -
INPATIENT

Date/Time

7/23/2025 18:54

Shift

Night

Reason for Admission

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure	147 mmHg
Diastolic Blood Pressure	90 mmHg
Blood Pressure	147 / 90
Heart Rate	114 beats/min
Temperature	97.3 °F
Respirations	18 brth/min
Height	70 in
Weight	202 lb
SPO2	98 %

****RN: Orders must be renewed daily**

Special Interventions This Shift **None**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation	Every 15 minutes
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Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

PAIN

Pain today? **No**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Is the Patient already on Suicide Precautions? **No**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

6. Have you ever done anything, started to do anything, or prepared to do anything to end your life? **No**

Ex: Collected pills, obtained gun, gave away valuables, wrote will/suicide note, took out pills but didn't swallow, held gun but changed mind or it was grabbed from your hand, went to roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

The items are then summed to
produce a total score.

The risk rating is determined based
on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	0

Record of aggression: During the
previous 24 hours, has the patient
behaved aggressively in any of the
following ways?

Physical aggression against
OBJECTS **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Verbal aggression against OTHER
PEOPLE **No**

Physical Aggression against OTHER
PEOPLE **No**

DASA

If patient scored Moderate or High
Risk and not on Aggression/Assault
Precautions notify MD

CURRENT STATUS

L.O.C.	Alert
Mobility	WNL
Activity	Normal
Awareness	Oriented x 4
Behavior	In bed
Mood	Euthymic
Affect	WNL
Thought Process	Organized
Thought Content	WNL
Speech	Normal
Detox Patient	No
ADLs	Taking care of ADLs
Bathed/Showered	Yes
Appearance	Neat/Clean

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Sleep **Slept throughout night**

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Supplements **N/A**

Elimination **Reports no problem**

REVIEW OF SYSTEMS

Review of Systems **WNL**

PATIENT EDUCATION

Topic **safety**

Level of Understanding **Partially understands Needs additional support for understanding**

Nursing Shift Narrative **Pt is A&O x 3, visible in the milieu, social with select peers. Pt is compliant with medications and meals. Pt denied any pain/constipation. Ativan PRN, no pending labs. Pt slept throughout the night, no SOB discomfort/ no unsafe behavior noted /reported. ALL level of observation and safety checks maintained, will continue to monitor, and report any changes.**

****RN: Document to the treatment plan every shift!**

PATIENT EDUCATION

- Conduct group (art, music, animal, movement, expressive) and monitor patient progress for the development of new leisure skills to decrease danger to others, homicidal ideation, and/or physical aggression (Intervention)

Nurse Signature **e-Signed by Yedena-Wassoua, Loudouba, RN at 07/24/2025 07:39**

Progress Note

Observation Date/Time:

NURSING PROGRESS NOTE -
INPATIENT

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Date/Time 7/24/2025 17:39

Shift Night

Reason for Admission States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure	161 mmHg
Diastolic Blood Pressure	90 mmHg
Blood Pressure	161 / 90
Heart Rate	100 beats/min
Temperature	97.8 °F
Respirations	18 brth/min
Height	70 in
Weight	202 lb
SPO2	98 %

****RN: Orders must be renewed daily**

Special Interventions This Shift **PRN Medications**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation **Every 15 minutes**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

PAIN

Pain today? **No**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Is the Patient already on Suicide Precautions? **Yes**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

6. Have you ever done anything, started to do anything, or prepared to do anything to end your life? **No**

Ex: Collected pills, obtained gun, gave away valuables, wrote will/suicide note, took out pills but didn't swallow, held gun but changed mind or it was grabbed from your hand, went to roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

The items are then summed to
produce a total score.

The risk rating is determined based
on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(1) - The patient has been impulsive and unpredictable in his/her affect or behavior or was quick to (over-) react to real and imagined slights, insults, and disappointments.
Unwillingness to Follow Directions	(1) - The patient has become angry and/or aggressive she/he was asked adhere to some aspect of her/his treatment or to the ward's routine in the last 24 hours.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	2

Record of aggression: During the
previous 24 hours, has the patient
behaved aggressively in any of the
following ways?

Physical aggression against
OBJECTS **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Verbal aggression against OTHER PEOPLE **No**

Physical Aggression against OTHER PEOPLE **No**

DASA

If patient scored Moderate or High Risk and not on Aggression/Assault Precautions notify MD

CURRENT STATUS

L.O.C.	Alert
Mobility	WNL
Activity	Restless and Pacing
Awareness	Aware of surroundings
Behavior	Non-compliant
Mood	Angry, Anxious and Irritable
Affect	Labile and Flat
Thought Process	Disorganized and Preoccupied
Thought Content	Delusional
Speech	Normal
Detox Patient	No
ADLs	Taking care of ADLs
Bathed/Showered	No

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Appearance	Disheveled
Sleep	Slept throughout night

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Supplements	N/A
Elimination	Reports no problem

REVIEW OF SYSTEMS

Review of Systems	WNL
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PATIENT EDUCATION

Topic	Safety
Level of Understanding	Understood
Nursing Shift Narrative	Patient presents with an irritable mood stating that he wants to go home and he has been rude, demanding and yelling at staff. Patient is visible in the milieu pacing down the hall way, Pt observed making several calls to his parents. Accepted scheduled medication and utilized Ativan 0.5 mg with fair effect. No assault, SI or seizures observed or reported this shift. Q 15 mins check maintained for safety.

**RN: Document to the treatment plan every shift!

PATIENT EDUCATION

- .Nursing will when Dallas identified with potential loss of control, ask Dallas what PRN medication is most helpful and administer medication according to doctor's order.

(Intervention)

- Nursing staff to administer medication and treatment(s) as ordered per MD (Intervention)
- Nursing will administer nicotine replacement protocol as ordered per MD (Intervention)
- .Staff will monitor patient as ordered by physician (Intervention)

Nurse Signature	e-Signed by Chileshe, Chikwanda at 07/24/2025 18:54
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Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Progress Note

Observation Date/Time:

NURSING PROGRESS NOTE -
INPATIENT

Date/Time **7/24/2025 23:52**

Shift **Night**

Reason for Admission **States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure	161 mmHg
Diastolic Blood Pressure	90 mmHg
Blood Pressure	161 / 90
Heart Rate	100 beats/min
Temperature	97.8 °F
Respirations	18 brth/min
Height	70 in
Weight	202 lb
SPO2	98 %

****RN: Orders must be renewed daily**

Special Interventions This Shift **None**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation	Every 15 minutes
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Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

PAIN

Pain today? **No**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Is the Patient already on Suicide Precautions? **No**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

6. Have you ever done anything, started to do anything, or prepared to do anything to end your life? **No**

Ex: Collected pills, obtained gun, gave away valuables, wrote will/suicide note, took out pills but didn't swallow, held gun but changed mind or it was grabbed from your hand, went to roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

The items are then summed to
produce a total score.

The risk rating is determined based
on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	0

Record of aggression: During the
previous 24 hours, has the patient
behaved aggressively in any of the
following ways?

Physical aggression against
OBJECTS **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Verbal aggression against OTHER
PEOPLE **No**

Physical Aggression against OTHER
PEOPLE **No**

DASA

If patient scored Moderate or High
Risk and not on Aggression/Assault
Precautions notify MD

CURRENT STATUS

L.O.C. **Asleep**

Mobility **asleep**

Activity **Asleep**

Awareness **Asleep**

Behavior **In bed**

Mood **asleep**

Affect **asleep**

Thought Process **asleep**

Thought Content **asleep**

Speech **asleep**

Detox Patient **No**

ADLs **Asleep**

Bathed/Showered **No**

Appearance **Neat/Clean and Asleep**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Sleep **Slept throughout night**

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Supplements **N/A**

Elimination **Asleep**

REVIEW OF SYSTEMS

Review of Systems **WNL**

PATIENT EDUCATION

Topic **safety**

Level of Understanding **Partially understands Needs additional support for understanding**

Nursing Shift Narrative **When this writer took over this patient care, pt was already in bed appears to be sleeping. Pt slept throughout the night, no SOB, no pain or any discomfort/ no unsafe behavior noted /reported. ALL level of observation and safety checks maintained, will continue to monitor and report any changes.**

****RN: Document to the treatment plan every shift!**

PATIENT EDUCATION

- Patient will, prompted or independently, use a self-identified regulation skill to safely manage danger to others, homicidal ideation, and/or physical aggression. (Objective)

Nurse Signature **e-Signed by Yedena-Wassoua, Loudouba, RN at 07/25/2025 07:12**

Progress Note

Observation Date/Time:

**NURSING PROGRESS NOTE -
INPATIENT**

Date/Time **7/25/2025 17:16**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Shift

Day

Reason for Admission

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Patient refused vital signs **Yes**

Weight (Q Wednesday) **0 lb**

****RN: Orders must be renewed daily**

Special Interventions This Shift **PRN Medications**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation **Every 15 minutes**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

PAIN

Pain today? **No**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Is the Patient already on Suicide Precautions? **Yes**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

6. Have you ever done anything, started to do anything, or prepared to do anything to end your life? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Ex: Collected pills, obtained gun, gave away valuables, wrote will/suicide note, took out pills but didn't swallow, held gun but changed mind or it was grabbed from your hand, went to roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Assessing the risk of aggression, Each item is scored for its presence (1), or absence (0), in the previous 24 hours.

The items are then summed to produce a total score.

The risk rating is determined based on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.

Negative Attitudes (0) - No negative attitudes

Verbal Threats (0) - The patient has not been verbally aggressive.

Total Score 0

Record of aggression: During the previous 24 hours, has the patient behaved aggressively in any of the following ways?

Physical aggression against OBJECTS No

Verbal aggression against OTHER PEOPLE No

Physical Aggression against OTHER PEOPLE No

DASA

If patient scored Moderate or High Risk and not on Aggression/Assault Precautions notify MD

CURRENT STATUS

L.O.C. Alert

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Mobility	WNL
Activity	Normal
Awareness	Aware of surroundings
Behavior	Attending groups
Mood	Angry, Anxious and Irritable
Affect	Labile
Thought Process	Preoccupied
Thought Content	WNL
Speech	Normal
Detox Patient	No
ADLs	Not taking care of ADLs
Bathed/Showered	No
Appearance	Disheveled
Sleep	up during the day

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Meals	Eating off unit
Supplements	N/A
Elimination	Reports no problem

REVIEW OF SYSTEMS

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Review of Systems

WNL

PATIENT EDUCATION

Topic

Safety

Level of Understanding

Understood

Nursing Shift Narrative

Patient appears to be calm this this shift . Reported sleeping good at night. Patient is visible in the milieu and attended groups. Patient eating meals in the cafeteria without any issues. Accepted scheduled medication and utilized nicotine gum. No assault, SI or seizures observed or reported this shift. Q 15 mins check maintained for safety.

**RN: Document to the treatment plan every shift!

PATIENT EDUCATION

- .Nursing will when Dallas identified with potential loss of control, ask Dallas what PRN medication is most helpful and administer medication according to doctor's order.
(Intervention)

- Nursing staff to administer medication and treatment(s) as ordered per MD (Intervention)
- Nursing will administer nicotine replacement protocol as ordered per MD (Intervention)
- .Staff will monitor patient as ordered by physician (Intervention)

Nurse Signature

e-Signed by Chileshe, Chikwanda at 07/26/2025 11:44

Progress Note

Observation Date/Time:

NURSING PROGRESS NOTE -
INPATIENT

Date/Time

7/26/2025 00:38

Shift

Night

Reason for Admission

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure	141 mmHg
Diastolic Blood Pressure	93 mmHg
Heart Rate	111 beats/min
Temperature	98.1 °F
Respirations	18 brth/min
SPO2	96 %

****RN: Orders must be renewed daily**

Special Interventions This Shift **None**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation	Every 15 minutes
Current Precautions	LOW Suicide, Seizure, Assault/Aggression

PAIN

Pain today? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Is the Patient already on Suicide Precautions? **No**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

6. Have you ever done anything,
started to do anything, or prepared to
do anything to end your life? **No**

Ex: Collected pills, obtained gun,
gave away valuables, wrote
will/suicide note, took out pills but
didn't swallow, held gun but changed
mind or it was grabbed from your
hand, went to roof but didn't jump; or
actually took pills, tried to shoot
yourself, cut yourself, tried to hang
yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate
Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the
patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the
patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

The items are then summed to produce a total score.

The risk rating is determined based on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	0

Record of aggression: During the previous 24 hours, has the patient behaved aggressively in any of the following ways?

Physical aggression against OBJECTS **No**

Verbal aggression against OTHER PEOPLE **No**

Physical Aggression against OTHER PEOPLE **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

DASA

If patient scored Moderate or High
Risk and not on Aggression/Assault
Precautions notify MD

CURRENT STATUS

L.O.C.	Asleep
Mobility	asleep
Activity	Asleep
Awareness	Asleep and asleep
Behavior	asleep
Mood	asleep
Affect	asleep
Thought Process	asleep
Thought Content	asleep
Speech	asleep
Detox Patient	No
ADLs	Asleep
Bathed/Showered	No
Appearance	Asleep
Sleep	Slept throughout night

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Supplements **N/A**

Elimination **Asleep**

REVIEW OF SYSTEMS

Review of Systems **WNL**

PATIENT EDUCATION

Topic **safety**

Level of Understanding **Partially understands Needs additional support for understanding**

Nursing Shift Narrative **Resumed care for this patient at 11 pm, pt was already in bed appears to be sleeping. Pt slept throughout the night, no SOB, no pain or any discomfort/ no unsafe behavior noted /reported. ALL level of observation and safety checks maintained, will continue to monitor and report any changes.**

****RN: Document to the treatment plan every shift!**

PATIENT EDUCATION

- Conduct group (art, music, animal, movement, expressive) and monitor patient progress for the development of new leisure skills to decrease danger to others, homicidal ideation, and/or physical aggression (Intervention)

Nurse Signature **e-Signed by Yedena-Wassoua, Loudouba, RN at 07/26/2025 05:25**

Progress Note

Observation Date/Time:

**NURSING PROGRESS NOTE -
INPATIENT**

Date/Time **7/26/2025 15:52**

Shift **Day**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Reason for Admission

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

****RN:** Orders must be renewed daily

Special Interventions This Shift **None**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation **Every 15 minutes**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

PAIN

Pain today? **No**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Is the Patient already on Suicide Precautions? **Yes**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

6. Have you ever done anything, started to do anything, or prepared to do anything to end your life? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Ex: Collected pills, obtained gun, gave away valuables, wrote will/suicide note, took out pills but didn't swallow, held gun but changed mind or it was grabbed from your hand, went to roof but didn't jump; or actually took pills, tried to shoot yourself, cut yourself, tried to hang yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Assessing the risk of aggression, Each item is scored for its presence (1), or absence (0), in the previous 24 hours.

The items are then summed to produce a total score.

The risk rating is determined based on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	0

Record of aggression: During the previous 24 hours, has the patient behaved aggressively in any of the following ways?

Physical aggression against OBJECTS	No
Verbal aggression against OTHER PEOPLE	No
Physical Aggression against OTHER PEOPLE	No

DASA

If patient scored Moderate or High Risk and not on Aggression/Assault Precautions notify MD

CURRENT STATUS

L.O.C. Alert

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Mobility	WNL
Activity	Normal
Awareness	Aware of surroundings
Behavior	Attending groups
Mood	Anxious and Irritable
Affect	Labile
Thought Process	Organized
Thought Content	WNL
Speech	Normal
Detox Patient	No
ADLs	Taking care of ADLs
Bathed/Showered	No
Appearance	Neat/Clean
Sleep	Awake during the day.

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Meals	Eating off unit
Supplements	N/A
Elimination	Reports no problem

REVIEW OF SYSTEMS

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Review of Systems

WNL

PATIENT EDUCATION

Topic

Safety

Level of Understanding

Understood

Nursing Shift Narrative

Patient appears to be calm this this shift . Reported sleeping good at night. Patient is visible in the milieu and attended groups. Patient eating meals in the cafeteria without any issues. Accepted scheduled medication and utilized prn Ativan 0.5mg and nicotine gum. No assault, SI or seizures observed or reported this shift. Q 15 mins check maintained for safety.

**RN: Document to the treatment plan every shift!

PATIENT EDUCATION

- Conduct group (art, music, animal, movement, expressive) and monitor patient progress for the development of new leisure skills to decrease danger to others, homicidal ideation, and/or physical aggression (Intervention)
- .Nursing will when Dallas identified with potential loss of control, ask Dallas what PRN medication is most helpful and administer medication according to doctor's order. (Intervention)
- Nursing will administer nicotine replacement protocol as ordered per MD (Intervention)
- .Staff will monitor patient as ordered by physician (Intervention)

Nurse Signature

e-Signed by Chileshe, Chikwanda at 07/26/2025 20:01

Progress Note

Observation Date/Time:

NURSING PROGRESS NOTE -
INPATIENT

Date/Time

7/27/2025 02:55

Shift

Night

Reason for Admission

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure **128 mmHg**

Diastolic Blood Pressure **90 mmHg**

Heart Rate **98 beats/min**

Temperature **98.0 °F**

Respirations **18 brth/min**

SPO2 **97 %**

****RN: Orders must be renewed daily**

Special Interventions This Shift **None**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation **Every 15 minutes**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

PAIN

Pain today? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Is the Patient already on Suicide Precautions? **No**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

6. Have you ever done anything,
started to do anything, or prepared to
do anything to end your life? **No**

Ex: Collected pills, obtained gun,
gave away valuables, wrote
will/suicide note, took out pills but
didn't swallow, held gun but changed
mind or it was grabbed from your
hand, went to roof but didn't jump; or
actually took pills, tried to shoot
yourself, cut yourself, tried to hang
yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate
Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the
patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the
patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

The items are then summed to produce a total score.

The risk rating is determined based on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	0

Record of aggression: During the previous 24 hours, has the patient behaved aggressively in any of the following ways?

Physical aggression against OBJECTS **No**

Verbal aggression against OTHER PEOPLE **No**

Physical Aggression against OTHER PEOPLE **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

DASA

If patient scored Moderate or High
Risk and not on Aggression/Assault
Precautions notify MD

CURRENT STATUS

L.O.C.	Asleep
Mobility	WNL
Activity	Asleep
Awareness	Asleep
Behavior	In bed
Mood	Euthymic
Affect	WNL
Thought Process	Organized
Thought Content	WNL
Speech	Normal
Detox Patient	No
ADLs	Asleep
Bathed/Showered	No
Appearance	Neat/Clean and Asleep
Sleep	Awake throughout night

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Supplements N/A

Elimination Asleep

REVIEW OF SYSTEMS

Review of Systems WNL

PATIENT EDUCATION

Topic Safety

Level of Understanding Understood

Nursing Shift Narrative
Pt is visible on the unit, social with staff and peers, pleasant upon approach. Attended nursing group with full participation, compliant with scheduled medications, utilizing prn Ativan 0.5mg. Expressed frustration and increased anxiety surrounding pending discharge, per pt. Pt reports he is discharging Tuesday and is becoming restless having to wait. Encouraged pt to reframe thinking to "3 sleep overs". Pt was appreciative of 1:1 support stating "That makes me feel better, to think of it that way, that's manageable". Pt denies SI/SIB/HI/AH/VH at this time, feels safe on the unit. No s/sx of seizure activity observed or reported.

Pt is observed resting quietly in assigned bed; eyes closed, respirations even and unlabored, appears to be asleep.

Pt is maintained on 15-minute observation checks for safety, no safety concerns at this time. Continue to monitor for safety, report any acute changes in medical/mental status to on call provider and provide support as needed.

**RN: Document to the treatment plan every shift!

PATIENT EDUCATION

- .Staff will monitor patient as ordered by physician (Intervention)

Nurse Signature e-Signed by Pizzi, Carolyn at 07/27/2025 06:20

Progress Note

Observation Date/Time:

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

NURSING PROGRESS NOTE -
INPATIENT

Date/Time **7/27/2025 17:59**

Shift **Day**

Reason for Admission **States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure **142 mmHg**

Diastolic Blood Pressure **60 mmHg**

Heart Rate **77 beats/min**

Temperature **98.7 °F**

Respirations **18 brth/min**

SPO2 **97 %**

****RN: Orders must be renewed daily**

Special Interventions This Shift **PRN Medications**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation **Every 15 minutes**

Current Precautions **LOW Suicide, Seizure, Assault/Aggression**

PAIN

Pain today? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Is the Patient already on Suicide Precautions? **No**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

6. Have you ever done anything,
started to do anything, or prepared to
do anything to end your life? **No**

Ex: Collected pills, obtained gun,
gave away valuables, wrote
will/suicide note, took out pills but
didn't swallow, held gun but changed
mind or it was grabbed from your
hand, went to roof but didn't jump; or
actually took pills, tried to shoot
yourself, cut yourself, tried to hang
yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate
Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the
patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the
patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

The items are then summed to produce a total score.

The risk rating is determined based on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(1) - The patient is easily annoyed or angered and unable to tolerate the presence of others.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(1) - The patient has become angry and/or aggressive she/he was asked adhere to some aspect of her/his treatment or to the ward's routine in the last 24 hours.
Sensitive to Perceived Provocation	(1) - The patient has tended to see others' actions as deliberate and harmful. She/he may misinterpret other people's behavior or respond with anger in a disproportionate manner to the extent of provocation. She/he has been been orickly, overly sensitive, and quick(1) to anger.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acception when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acception that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.

Total Score 3

Record of aggression: During the previous 24 hours, has the patient behaved aggressively in any of the following ways?

Physical aggression against OBJECTS No

Verbal aggression against OTHER PEOPLE No

Physical Aggression against OTHER PEOPLE No

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

DASA

If patient scored Moderate or High
Risk and not on Aggression/Assault
Precautions notify MD

CURRENT STATUS

L.O.C.	Alert
Mobility	WNL
Activity	Normal
Awareness	Aware of surroundings
Behavior	Attending groups
Mood	Anxious and Irritable
Affect	Labile
Thought Process	Disorganized
Thought Content	Delusional
Speech	Normal
Detox Patient	No
ADLs	Taking care of ADLs
Bathed/Showered	No
Appearance	Neat/Clean
Sleep	Napped during the day

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

DIETARY INTAKE

Meals	Eating off unit
Breakfast	100%
Lunch	100%
Dinner	100%
Snack	Yes
Supplements	N/A
Elimination	Reports no problem

REVIEW OF SYSTEMS

Review of Systems	WNL
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PATIENT EDUCATION

Topic	Safety
Level of Understanding	Partially understands Needs additional support for understanding
Nursing Shift Narrative	Patient presented with a flat affect, alert and verbally responsive. Denies having any pain or discomfort. Able to make his needs known. Noticed interacting well with selective peers. Positive for all his meds as ordered. Prn gum 2 mg . Joined the group meeting and coloring. Transited well at the cafe. He has good appetite. Staff educated patient how to use his coping skills. No issues reported this shift. will continue on 15 mins checks for safety.

**RN: Document to the treatment plan every shift!

PATIENT EDUCATION

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

- .Nursing will when Dallas identified with potential loss of control, ask Dallas what PRN medication is most helpful and administer medication according to doctor's order.

(Intervention)

- Nursing staff to administer medication and treatment(s) as ordered per MD (Intervention)
- Nursing will administer nicotine replacement protocol as ordered per MD (Intervention)

Nurse Signature

e-Signed by Nalunkuma, Faridah, RN at 07/28/2025 18:52

Progress Note

Observation Date/Time:

NURSING PROGRESS NOTE -
INPATIENT

Date/Time

7/27/2025 22:57

Shift

Night

Reason for Admission

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure	143 mmHg
Diastolic Blood Pressure	92 mmHg
Heart Rate	100 beats/min
Temperature	98.2 °F
Respirations	18 brth/min
SPO2	98 %

****RN: Orders must be renewed daily**

Special Interventions This Shift	None
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Medical Interventions This Shift	None
----------------------------------	-------------

Risk Assessments

Current Level of Observation	Every 15 minutes
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Current Precautions	LOW Suicide, Seizure, Assault/Aggression
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PAIN

Pain today?	No
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Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Is the Patient already on Suicide Precautions? **No**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

6. Have you ever done anything,
started to do anything, or prepared to
do anything to end your life? **No**

Ex: Collected pills, obtained gun,
gave away valuables, wrote
will/suicide note, took out pills but
didn't swallow, held gun but changed
mind or it was grabbed from your
hand, went to roof but didn't jump; or
actually took pills, tried to shoot
yourself, cut yourself, tried to hang
yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate
Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the
patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the
patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

The items are then summed to produce a total score.

The risk rating is determined based on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	0

Record of aggression: During the previous 24 hours, has the patient behaved aggressively in any of the following ways?

Physical aggression against OBJECTS **No**

Verbal aggression against OTHER PEOPLE **No**

Physical Aggression against OTHER PEOPLE **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

DASA

If patient scored Moderate or High
Risk and not on Aggression/Assault
Precautions notify MD

CURRENT STATUS

L.O.C.	Alert
Mobility	WNL
Activity	Normal
Awareness	Oriented x 4
Behavior	In bed
Mood	Euthymic
Affect	WNL
Thought Process	Organized
Thought Content	WNL
Speech	Normal
Detox Patient	No
ADLs	Taking care of ADLs
Bathed/Showered	No
Appearance	Neat/Clean
Sleep	Slept throughout night

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Supplements **N/A**

Elimination **Reports no problem**

REVIEW OF SYSTEMS

Review of Systems **WNL**

PATIENT EDUCATION

Topic **safety**

Level of Understanding **Partially understands Needs additional support for understanding**

Nursing Shift Narrative **Pt is A&O x 3, visible in the milieu, social with select peers. Pt is compliant with medications and meals. Pt denied any pain/constipation. Ativan PRN, no pending labs. Pt slept throughout the night, no SOB discomfort/ no unsafe behavior noted /reported. ALL level of observation and safety checks maintained, will continue to monitor, and report any changes.**

****RN: Document to the treatment plan every shift!**

PATIENT EDUCATION

- Conduct group (art, music, animal, movement, expressive) and monitor patient progress for the development of new leisure skills to decrease danger to others, homicidal ideation, and/or physical aggression (Intervention)

Nurse Signature **e-Signed by Yedena-Wassoua, Loudouba, RN at 07/28/2025 06:57**

Progress Note

Observation Date/Time:

**NURSING PROGRESS NOTE -
INPATIENT**

Date/Time **7/28/2025 18:52**

Shift **Day**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Reason for Admission

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure	136 mmHg
Diastolic Blood Pressure	84 mmHg
Heart Rate	100 beats/min
Temperature	98.3 °F
Respirations	18 brth/min
SPO2	96 %

****RN: Orders must be renewed daily**

Special Interventions This Shift **PRN Medications**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation	Every 15 minutes
Current Precautions	LOW Suicide, Seizure, Assault/Aggression

PAIN

Pain today? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Is the Patient already on Suicide Precautions? **No**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

6. Have you ever done anything,
started to do anything, or prepared to
do anything to end your life? **No**

Ex: Collected pills, obtained gun,
gave away valuables, wrote
will/suicide note, took out pills but
didn't swallow, held gun but changed
mind or it was grabbed from your
hand, went to roof but didn't jump; or
actually took pills, tried to shoot
yourself, cut yourself, tried to hang
yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate
Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the
patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the
patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

The items are then summed to produce a total score.

The risk rating is determined based on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(1) - The patient is easily annoyed or angered and unable to tolerate the presence of others.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(1) - The patient has become angry and/or aggressive she/he was asked adhere to some aspect of her/his treatment or to the ward's routine in the last 24 hours.
Sensitive to Perceived Provocation	(1) - The patient has tended to see others' actions as deliberate and harmful. She/he may misinterpret other people's behavior or respond with anger in a disproportionate manner to the extent of provocation. She/he has been been orickly, overly sensitive, and quick(1) to anger.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acception when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acception that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	3

Record of aggression: During the previous 24 hours, has the patient behaved aggressively in any of the following ways?

Physical aggression against OBJECTS **No**

Verbal aggression against OTHER PEOPLE **No**

Physical Aggression against OTHER PEOPLE **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

DASA

If patient scored Moderate or High
Risk and not on Aggression/Assault
Precautions notify MD

CURRENT STATUS

L.O.C.	Alert
Mobility	WNL
Activity	Normal
Awareness	Aware of surroundings
Behavior	Attending groups
Mood	Anxious
Affect	Labile
Thought Process	Disorganized
Thought Content	Delusional
Speech	Normal
Detox Patient	No
ADLs	Taking care of ADLs
Bathed/Showered	Yes
Appearance	Neat/Clean
Sleep	Napped during the day

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

DIETARY INTAKE

Meals	Eating off unit
Breakfast	100%
Lunch	100%
Dinner	100%
Snack	Yes
Supplements	N/A
Elimination	Reports no problem

REVIEW OF SYSTEMS

Review of Systems	WNL
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PATIENT EDUCATION

Topic	Safety
Level of Understanding	Partially understands Needs additional support for understanding
Nursing Shift Narrative	Patient presented with a flat affect, alert and verbally responsive. Denies having any pain or discomfort. Able to make his needs known. Noticed interacting well with selective peers. Positive for all his meds as ordered. Prn gum 2 mg . Joined the group meeting and coloring. Transited well at the cafe. He has good appetite. Staff educated patient how to use his coping skills. No issues reported this shift. will continue on 15 mins checks for safety.

**RN: Document to the treatment plan every shift!

PATIENT EDUCATION

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

- .Nursing will when Dallas identified with potential loss of control, ask Dallas what PRN medication is most helpful and administer medication according to doctor's order.
(Intervention)

- Nursing staff to administer medication and treatment(s) as ordered per MD (Intervention)
- Nursing will administer nicotine replacement protocol as ordered per MD (Intervention)

Nurse Signature

e-Signed by Nalunkuma, Faridah, RN at 07/28/2025 18:55

Progress Note

Observation Date/Time:

NURSING PROGRESS NOTE -
INPATIENT

Date/Time

7/29/2025 02:04

Shift

Night

Reason for Admission

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: Adult A

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

History of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Diagnosis:

Primary: Bipolar Disorder

Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:

Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.

Psychoeducation and engagement with family and peer support systems.

Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

VITAL SIGNS

Systolic Blood Pressure	136 mmHg
Diastolic Blood Pressure	84 mmHg
Heart Rate	100 beats/min
Temperature	98.3 °F
Respirations	18 brth/min
SPO2	96 %

****RN: Orders must be renewed daily**

Special Interventions This Shift **None**

Medical Interventions This Shift **None**

Risk Assessments

Current Level of Observation	Every 15 minutes
Current Precautions	LOW Suicide, Seizure, Assault/Aggression

PAIN

Pain today? **No**

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

ELOPEMENT RISK

Is Elopement currently a part of the patient's presentation? **No**

SEXUAL ACTING OUT

Is Sexually Acting out currently a part of the patient's presentation? **No**

Moderate Risk: RN to consult MD for SAO precaution and discuss any additional necessary orders or practices of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes, and/or room assignment on the unit. -

High Risk (15+): RN to consult MD for SAO precaution and MD will order at least one of the following: blocked room/"no roommate" status, "distance rule" from peers, observation level changes

FALL RISK

Are falls currently a part of the patient's presentation? **No**

SUICIDALITY/C-SSRS RECENT

1. Have you wished you were dead or wished you could go to sleep and not wake up? **No**

2. Have you actually had any thoughts of killing yourself? **No**

Is the Patient already on Suicide Precautions? **No**

If patient answers Yes to Either Question 1 or Question 2 AND is NOT already on Suicide Precautions, CONTACT PROVIDER to review

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

PCU: **Adult A**

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

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6. Have you ever done anything,
started to do anything, or prepared to
do anything to end your life? **No**

Ex: Collected pills, obtained gun,
gave away valuables, wrote
will/suicide note, took out pills but
didn't swallow, held gun but changed
mind or it was grabbed from your
hand, went to roof but didn't jump; or
actually took pills, tried to shoot
yourself, cut yourself, tried to hang
yourself, etc.

Only Yes to 1 or 2 - Low Risk

Only Yes to 1 or 2 and 3 Moderate
Risk

Yes to 4, 5, or 6 High Risk

Risk Level **Low Risk**

SELF-HARM

Is self-harm currently a part of the
patient's presentation? **No**

VULNERABILITY

Is Vulnerability currently a part of the
patient's presentation? **No**

DYNAMIC APPRAISAL OF SITUATIONAL AGGRESSION (DASA)

Assessing the risk of aggression,
Each item is scored for its presence
(1), or absence (0), in the previous 24
hours.

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

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The items are then summed to produce a total score.

The risk rating is determined based on total score.

0 = Low Risk

1 - 3 = Moderate Risk

4 - 7 = High Risk

Assessment

Irritability	(0) - The patient has been calm, tolerant and relaxed. She/he is comfortable and relaxed in the company of other patients with staff.
Impulsivity	(0) - The patient has been affectively and/or behaviorally stable and composed.
Unwillingness to Follow Directions	(0) - The patient in the last 24 hours has been compliant with any requests and directions.
Sensitive to Perceived Provocation	(0) - In the last 24 hours the patient has not become extraordinarily angry or seen everything that occurs around her/him as provocative. She/he is not 'overly sensitive' or provocative'.
Easily Angered when Requests are Denied	(0) - The patient has been calm and acceptance when she/he is asked to wait whilst her/his request is attended to. She/he has been understanding and acceptance that her/his request has been unable to be fulfilled at that time.
Negative Attitudes	(0) - No negative attitudes
Verbal Threats	(0) - The patient has not been verbally aggressive.
Total Score	0

Record of aggression: During the previous 24 hours, has the patient behaved aggressively in any of the following ways?

Physical aggression against OBJECTS **No**

Verbal aggression against OTHER PEOPLE **No**

Physical Aggression against OTHER PEOPLE **No**

Nursing Progress Note

Flaherty, Dallas G

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DOB: **11/03/1994**

Admit Date: **07/22/2025**

DASA

If patient scored Moderate or High
Risk and not on Aggression/Assault
Precautions notify MD

CURRENT STATUS

L.O.C.	Alert
Mobility	WNL
Activity	Asleep
Awareness	Aware of surroundings
Behavior	In bed and calm
Mood	Euthymic
Affect	Flat
Thought Process	Disorganized
Thought Content	WNL
Speech	Normal
Detox Patient	No
ADLs	Taking care of ADLs
Bathed/Showered	No
Appearance	Neat/Clean
Sleep	Slept throughout night

REVIEW OF SYSTEMS/INTAKE AND OUTPUT

DIETARY INTAKE

Nursing Progress Note

Flaherty, Dallas G

Westborough Behavioral Health Hospital

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Admit Date: **07/22/2025**

Snack	Yes
Supplements	N/A
Elimination	Reports no problem and Asleep

REVIEW OF SYSTEMS

Review of Systems	WNL
-------------------	------------

PATIENT EDUCATION

Topic	coping strategies
-------	--------------------------

Level of Understanding	Partially understands Needs additional support for understanding
------------------------	---

Nursing Shift Narrative	Patient visible on the unit during change of shift conversing with his peers. No exhibition of aggression/assault. Mood calm with flat affect. Denies SI and q15 min checks maintained for safety and location. Sleeping throughout the night with eyes closed and active respiration.
-------------------------	---

****RN: Document to the treatment plan every shift!**

PATIENT EDUCATION

- Conduct group (art, music, animal, movement, expressive) and monitor patient progress for the development of new leisure skills to decrease danger to others, homicidal ideation, and/or physical aggression (Intervention)
- Patient will, prompted or independently, use a self-identified regulation skill to safely manage suicidal ideation and/or danger to self. (Objective)
- Dallas will take medications as ordered (Objective)
- Dallas will practice an identified coping skill to manage/reduce smoking urges (Objective)

Nurse Signature	e-Signed by Gutheru, Annie at 07/29/2025 06:46
-----------------	---

Psychiatric Evaluation

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time: **7/23/2025 9:16:07AM**

Initial

Date of Service:

7/23/2025 00:00

Chief Complaint (Patient's own words)

States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."

Psychiatric Evaluation

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/23/2025 9:16:07AM

History Of Present Illness

Presenting Problem:

Patient was taken to the emergency department under Section 12 following homicidal statements directed toward his parents. He denies current homicidal ideation but admits to past threats, including an intent to punch his father. He reports frustration stemming from unresolved family disputes, notably regarding his uncle's coin collection, which his father is withholding. He attempted to file a restraining order against his mother but was unable to articulate the specific reasons.

Psychiatric History:

Diagnosed with Bipolar Disorder, Autism Spectrum Disorder (ASD), Generalized Anxiety Disorder (GAD), and history of Traumatic Brain Injury (TBI) from birth complications.

Has a history of suicidal ideation and self-injurious behavior, including choking himself with a belt during adolescence.

No prior inpatient psychiatric admissions.

Non-compliant with psychiatric medications over the past few months, citing a preference for a holistic approach.

Behavioral Observations:

Appearance: Calm

Mood: Anxious

Affect: Congruent

Behavior: Cooperative

Speech: Clear

Thought Process: Circumstantial

Thought Content: No current suicidal or homicidal ideation

Orientation: Oriented to person, place, time, and situation

Insight: Poor

Judgment: Impaired

Concentration: Easily distracted

Family and Social History:

Reports a poor relationship with his father; primarily raised by his mother.

Engaged; has no children.

Support system includes family members and friends.

Substance Use History:

Occasional alcohol consumption ("few beers here and there")

Cannabis use

Family history of alcohol abuse (father)

Protective Factors:

Internal: Recognizes reasons for living

External: Beloved pet cats and a supportive social network of family and friends

Assessment Summary:

Patient appeared pressured in speech during the interview but remained cooperative. He repeatedly stated he was "fine" and expressed a desire to return home to care for his cats. His mother reported escalating behavioral issues over recent months, including verbal aggression and property destruction, raising concerns about his safety. Patient will benefit from inpatient psychiatric admission for stabilization, medication assessment, and comprehensive aftercare planning.

Psychiatric Evaluation

Flaherty, Dallas G

MR#: 109931
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Diagnosis:
Primary: Bipolar Disorder
Secondary: Autism Spectrum Disorder (ASD)

Treatment Plan:
Admit to inpatient psychiatric unit for safety, stabilization, and medication evaluation.
Psychoeducation and engagement with family and peer support systems.
Develop an individualized aftercare plan, including outpatient follow-up scheduled for 08/08/2025.

Psychiatric History see above

Has the patient been in treatment previously? No

Patient admits to substance abuse Yes

Substance Use

Row	Used in last 30 Days	Age of 1st Use	Current Amount	Current Frequency	Current Duration	Date of Last Use/Amt	Route of Usage (IV, Smoking, etc.)
1	Nicotine	18	1.5 to 2 packs daily	Daily	12 years	Prior to ED Admittance	Inhalation
2	Cannabis	18	0.5 ounce daily	Daily	12 years	Prior to ED Admittance	Inhalation
3	ETOH	Unknown	Twice weekly: 6 beers/ 2 shots	Twice weekly	Unknown	Unknown	PO

Initial

Legal Status Conditional Voluntary

Abuse/Neglect/Trauma

Abuse/Neglect/Trauma

Medical

Psychiatric Evaluation

Flaherty, Dallas G

MR#: 109931
Hosp #: 2501689
DOB: 11/03/1994
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Observation Date/Time: 7/23/2025 9:16:07AM

Medical History

Bipolar, Seizures, Suicide attempts and "Bipolar disorder, current episode manic without psychotic features, severe, Autistic disorder, Generalized anxiety disorder, Personal history of traumatic brain injury, Alcohol abuse, uncomplicated, Attention-deficit hyperactivity disorder, unspecified type, Cannabis abuse, uncomplicated, Epilepsy, unspecified, not intractable, without status epilepticus, Essential (primary) hypertension, Nicotine dependence, unspecified, uncomplicated, Pervasive developmental disorder, unspecified, Presence of other specified devices"

Developmental/Educational / Vocational History

Developmental/Educational / Vocational History unk

Military

MILITARY HISTORY

Is the patient active or former military? No

Social

Current Living Situation Mother-Terry

Family Relationships Parents and fiance. New OP therapist appt booked on 8/8/25.

Social History / Support Networks Family

Family History of Suicide/Mental Health/ Substance Abuse Yes

Family hx of bipolar, anxiety, depression and SUD.

Assessment

Allergies

Assessment

Allergy Comments	Type	Reaction	Severity	Date	Code	System
metoprolol	Drug Allergy	-----	-----	----	----	d00298-----MULTUM
NO FOOD ALLERGIES	Drug Allergy					

Psychiatric Evaluation

Flaherty, Dallas G

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Home Medications and Admission Med Rec Review

Psychiatric Evaluation

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/23/2025 9:16:07AM

Home Meds:

Description	Brand	Dose	Route	Frequency	PRN
Indication	-----	-----	-----	-----	---
Although a patient record was found, no medications were available.					
amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY	
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
divalproex sodium Oral 500 mg ERT Epilepsy, unspecified, not (Once-Daily) intractable, without status	Depakote ER	500 mg	PO	DAILY	
epilepticus					
Notes:Once-Daily FLUoxetine 40 mg CAP Bipolar disorder, current	PROzac	40 mg	PO	DAILY	
episode manic without					
psychotic features, severe					
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY	
hypertension					
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P
disorder					
Notes:SALAD omeprazole Oral 20 mg DRC Notes:SALAD	PriLOSEC	20 mg	PO	DAILY	GERD

Admission Meds:

Description	Brand	Dose	Route	Frequency	PRN
Indication	-----	-----	-----	-----	---
amLODIPine Oral 10 mg TAB Essential (primary)	Norvasc	10 mg	PO	DAILY	
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
divalproex sodium Oral 500 mg ERT Epilepsy, unspecified, not (Once-Daily) intractable, without status	Depakote ER	500 mg	PO	DAILY	

Psychiatric Evaluation

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epilepticus						
Notes:Once-Daily						
FLUoxetine 40 mg CAP	PROzac	40 mg	PO	DAILY		
Bipolar disorder, current						
episode manic without						
psychotic features, severe						
hydroCHLORothiazide Oral 25 mg TAB	Oretic	25 mg	PO	DAILY		
Essential (primary)						
hypertension						
LORazepam Oral 0.5 mg TAB	Ativan	0.5 mg	PO	BID	P	
Generalized anxiety						
disorder						
Notes:SALAD						
omeprazole Oral 20 mg DRC	PriLOSEC	20 mg	PO	DAILY		GERD
Notes:SALAD						

Review of Systems

Constitutional	No Complaints
Eyes	No Complaints
ENMT	No Complaints
Cardiovascular	No Complaints
Respiratory	No Complaints
Gastrointestinal	No Complaints
Genitourinary	No Complaints
Musculoskeletal	Pain
Musculoskeletal Comments	Back pain
Integumentary/Breast	No Complaints
Neurological	No Complaints
Psychiatric	No Complaints

Psychiatric Evaluation

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Endocrine	No Complaints
Hematologic/Lymphatic	No Complaints
Allergic/Immunologic	No Complaints
Gait & Station	No deficits
Abnormal Movements	None present
Physical Complaints	Denies

Vital Signs

Systolic Blood Pressure	131 mmHg
Diastolic Blood Pressure	91 mmHg
Blood Pressure	120 / 74
Heart Rate	102 beats/min
Temperature	98 °F
Respirations	18 brth/min
SPO2	96 %

Mental Status Exam

General Appearance	Neat
Attitude/Behavior	Cooperative
Affect	Appropriate
Mood	Euthymic
Speech/Language: Quantity	Spontaneous
Speech/Language: Rate	Normal

Psychiatric Evaluation

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Speech/Language: Volume	Normal
Speech/Language: Rhythm	Clear
Thought Process	Circumstantial
Thought Content	Appropriate to situation
Suicidal	Denies
Homicidal/Violent	Denies
Perception: Hallucination(s)	Denies
Orientation	Person and Place
Attention Span	Intact
(Direct Questioning) How Tested/Assessed	Digit span
Recent Memory	Not Impaired
(Direct Questioning) How Tested/Assessed	Name
Remote Memory	Not Impaired
(Direct Questioning) How Tested/Assessed	Name of childhood town
Abstract Reasoning	Not Impaired
(Direct Questioning) How Tested/Assessed	Similarities
Intelligence	Average
How Tested/Assessed	Vocabulary
Judgement	Poor
How Tested/Assessed	Difficulty predicting results of choices
Insight	Poor
How Tested/Assessed	Patient inability to understand: nature of illness

Psychiatric Evaluation

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Suicidal Ideations **Patient currently denies**

Homicidal Ideations **Patient currently denies**

Suicidal Ideation - Since Last Contact

Ask questions 1 and 2. If both are negative, proceed to "Suicidal Behavior" section. If the answer to question 2 is "yes", ask questions 3, 4 and 5. If the answer to question 1 and/or 2 is "yes", complete "Intensity of Ideation" section below.

1. Wish to be Dead **No**

2. Non-Specific Active Suicidal Thoughts **No**

Are the answers to Question one (1) or Question two (2) yes? **No**

Suicidal Behavior

Actual Attempt:

A potentially self-injurious act committed with at least some wish to die, as a result of act. Behavior was in part thought of as method to kill oneself. Intent does not have to be 100%.

If there is any intent/desire to die associated with the act then it can be considered an actual suicide attempt. There does not have to be an injury or harm, just the potential for injury or harm. If person pulls trigger while gun is in mouth but gun is broken so no injury results,

this is considered an attempt.

Inferring Intent: Even if an individual denies intent/wish to die, it may be inferred clinically from the behavior or circumstances. For example, a highly lethal act that is clearly not an accident so no other intent but suicide can be inferred

(e.g. gunshot to head, jumping from window of a high floor/story). Also if someone denies intent to die, but they thought that what they did could be lethal, intent may be inferred.

Have you made a suicide attempt? **No**

Has subject engaged in Non-Suicidal Self-Injurious Behavior? **No**

Interrupted Attempt

When the person is interrupted (by an outside circumstance) from starting the potentially self-injurious act (if not for that, actual attempt would have occurred).

Overdose: Person has pills in hand but is stopped from ingesting.

Once they ingest any pills, this becomes an attempt rather than an interrupted attempt. Shooting: Person has gun pointed toward self, gun is taken away by someone else, or is somehow prevented from pulling trigger. Once they pull the trigger, even if the gun fails to fire, it is an attempt.

Jumping: Person is poised to jump, is grabbed and taken down from ledge. Hanging: Person has noose around neck but has not yet started to hang - is stopped from doing so.

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Has there been a time when you started to do something to end your life but someone or something stopped you before you actually did anything?

No

Aborted or Self-Interrupted Attempt

When person begins to take steps toward making a suicide attempt, but stops themselves before they actually have engaged in any self-destructive behavior. Examples are similar to interrupted attempts, except that the individual stops him/herself, instead of being stopped by something else.

Has there been a time when you started to do something to try to end your life but you stopped yourself before you actually did anything?

No

Preparatory Acts of Behavior

Acts or preparation towards imminently making a suicide attempt. This can include anything beyond a verbalization or thought, such as assembling a specific method. (e.g. buying pills, purchasing a gun) or preparing for one's death by suicide (e.g. giving things away, writing a suicide note).

Have you taken any steps towards making a suicide attempt or preparing to kill yourself (such as collecting pills, getting a gun, giving valuables away or writing a suicide note)?

No

Actual Lethality/Medical Damage

0. No physical damage or very minor physical damage (e.g., surface scratches).

1. Minor physical damage (e.g., lethargic speech; first-degree burns; mild bleeding; sprains).

2. Moderate physical damage; medical attention needed (e.g., conscious but sleepy, somewhat responsive, second-degree burns; bleeding of major vessel).

3. Moderately severe physical damage; medical hospitalization and likely intensive care (e.g., comatose with reflexes intact; third-degree burns less than 20% of body; extensive blood loss but can recover; major fractures).

4. Severe physical damage; medical hospitalization with intensive care required (e.g., comatose without reflexes; third-degree burns over 20% of body; extensive blood loss with unstable vital signs; major damage to a vital area)

Most Lethal Attempt Date

na

Enter Code

na

Potential Lethality: Only Answer if Actual Lethality = 0

Likely lethality of actual attempt if no medical damage (the following examples, while having no actual medical damage, had potential for very serious lethality;

put gun in mouth and pulled the trigger but gun fails to fire so no medical damage; laying on train tracks with oncoming train but pulled away before run over).

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Observation Date/Time:

0 = Behavior not likely to result in injury
1 = Behavior likely to result in injury but likely to cause death
2 = Behavior likely to result in death despite available medical care

Enter Code representing Most Lethal Attempt **na**

1.EXTREMITY MOVEMENTS

Stand	0=None
Upper	0=none
Lower	0=None
Trunk	0=None

2.FACIAL AND ORAL MOVEMENTS

Mouth	0=None
Muscles of Facial Expression	0=None
Lips and Peri-oral Area	0=None
Jaw	0=None
Tongue	0=None

3.GLOBAL JUDGEMENTS

Severity of abnormal movements	0=None
Incapacitation due to abnormal movements	0=None
Patient's awareness of abnormal movements	0=None, Normal

4.DENTAL STATUS

Any current problems with teeth and/or dentures?	0=No
--	-------------

Psychiatric Evaluation

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Does patient usually wear dentures? **0=No**

5.AIMS TOTAL

AIMS TOTAL: **0**

Assets/Strengths/Liabilities/Special Needs

Patient Assets and Strengths	Stable Housing, Supportive Family/Friends and Hx of Psychiatric Stability or Sobriety
------------------------------	--

Patient Liabilities and Special Needs	Lack of Motivation, Unemployed and Perseverating on mom/ blame shifting
---------------------------------------	--

Plan of Care

Problems to be addressed	Manic Symptomatology
--------------------------	-----------------------------

Dangerousness to	Self
------------------	-------------

Medical Problem(s)	see h and P
--------------------	--------------------

Short Term Goals	Decrease agitation/anxiety/panic and Stabilize on medication
------------------	---

Current Level of Observation	Every 15 minutes
------------------------------	-------------------------

Current Precautions	LOW Suicide, Seizure, Assault/Aggression
---------------------	---

Estimated Length of Stay (days)	14 Days
---------------------------------	----------------

Prognosis	good with mood stabilizer
-----------	----------------------------------

Initial Treatment Plan	Initial pharmacologic approach, "Psychiatric rounds to clarify diagnosis, manage medication" and Begin discharge planning for placement
------------------------	--

Preliminary Discharge Plan	Home
----------------------------	-------------

Diagnostic Formulation	see notes
------------------------	------------------

Initial Diagnosis	
-------------------	--

Psychiatric Evaluation

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Plan of Care

Diagnosis Code	System	Class	Type	Priority	Date
Bipolar disorder, current episode manic without psychotic features, severe	F31.13 ICD-10	Psychiatric	Provisional	Primary Diagnosis	
Autistic disorder	F84.0 ICD-10	Psychiatric	Working		
Generalized anxiety disorder	F41.1 ICD-10	Psychiatric	Working		
Personal history of traumatic brain injury	Z87.820 ICD-10	Medical	Working		
Alcohol abuse, uncomplicated	F10.10 ICD-10		Working		
Attention-deficit hyperactivity disorder, unspecified type	F90.9 ICD-10		Working		
Cannabis abuse, uncomplicated	F12.10 ICD-10		Working		
Epilepsy, unspecified, not intractable, without status epilepticus	G40.909 ICD-10		Working		
Essential (primary) hypertension	I10 ICD-10		Working		
Nicotine dependence, unspecified, uncomplicated	F17.200 ICD-10		Working		
Pervasive developmental disorder, unspecified	F84.9 ICD-10		Working		
Presence of other specified devices	Z97.8 ICD-10		Working		

ATTESTATION

I,

Okorafor, Ayodeji E, DNP

have reviewed and updated the above diagnoses as appropriate

Yes

INITIAL CERTIFICATION

Psychiatric Evaluation

Flaherty, Dallas G

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I certify that inpatient psychiatric hospital services furnished are necessary for either: (1) treatment which could reasonably be expected to improve the patient's condition, or (2) diagnostic study. **Yes**

I also certify that the patient continues to need, on a daily basis, active treatment furnished directly by or requiring the supervision of inpatient psychiatric facility personnel. **Yes**

Psychosocial Stressors **Social environment and Primary support system**

Plan of Care

Psychiatric Evaluation

Flaherty, Dallas G

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Medication Orders:

Description	Brand	Dose	Route	Frequency	PRN
amlodipine Oral 10 mg TAB Essential (primary) -----	Norvasc	10 mg	PO	DAILY	---
hypertension					
busPIRone Oral 10 mg TAB Generalized anxiety	BuSpar	10 mg	PO	BID	
disorder					
divalproex sodium Oral 500 mg ERT Epilepsy, unspecified, not (Once-Daily) intractable, without status	Depakote ER	500 mg	PO	DAILY	
epilepticus Notes: Once-Daily					
FLUoxetine Oral 20 mg CAP Bipolar disorder, current	PROzac	40 mg	PO	DAILY	
episode manic without					
psychotic features, severe					
hydroCHLORothiazide Oral 25 mg TAB Essential (primary)	Oretic	25 mg	PO	DAILY	
hypertension					
LORazepam Oral 0.5 mg TAB Generalized anxiety	Ativan	0.5 mg	PO	BID	P
disorder					
Notes: SALAD					
nicotine Oral Transmucosal 2 mg Smoking Cessation GUM	Nicorette	4 mg	OM	Q2H	P
nicotine 21 mg/24 hr Transdermal Nicotine Dependence TERF	*nicotine patch 21mg	1	TD	DAILY	P
omeprazole Oral 20 mg DRC Notes: SALAD	PriLOSEC	20 mg	PO	DAILY	GERD

Ancillary Orders:

Description	Frequency	PRN
-----	-----	---
-Adult Admit to: Conditional Voluntary Adult		
-COMPREHENSIVE METABOLIC PANEL	IN AM-UC	
-CBC (INCLUDES DIFF/PLT)	IN AM-UC	
-Assault/Aggression Precautions		
-Seizure Precautions		
- LOW Suicide Precaution		
-Certification of Admission		

Notes: I certify that this patient's inpatient psychiatric hospital admission is medically necessary for treatment which can reasonably be expected to improve the patient's condition and/ or diagnostic study

Psychiatric Evaluation

Flaherty, Dallas G

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DOB: 11/03/1994

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Observation Date/Time: 7/23/2025 9:16:07AM

-Community Meeting
-Therapeutic Recreation/Activity Therapy
-Psycho-educational Groups
-*Regular Diet
-History and Physical Exam
-Weight on admission and Weekly
-Vital Signs

WEEKLY
VS_BID_Ad
ult
ObsPre

-Review Precaution and Observation Orders

Notes:Nurse to review and confirm patient's precautions and observations and document in nursing notes

-Every 15 Minutes Observation

-Medical Diagnosis:

-Psychiatric Diagnosis:

-Psychotherapy Process Groups

-Dietary Consult: Consult - Nutrition screen score of 3 or More

Coordination of Care

clinical team

Is it appropriate to bill for this documentation?

Yes

90791 - Psychiatric Diagnostic Interview W/O Medical

Yes

I do hereby attest that this information is true, accurate and complete to the best of my knowledge and I understand that any falsification, omission, or concealment of material fact may subject me to administrative, civil, or criminal liability.

Signature

Psychiatrist Signature

e-Signed by Okorafor, Ayodeji E, DNP at 07/23/2025 11:47

Abuse/Neglect/Trauma

Psychiatric Evaluation

Flaherty, Dallas G

MR#: 109931

Hosp #: 2501689

DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/23/2025 9:16:07AM

Row	Yes/No	Victim/Perp	Documentation/T ime Frame of Occurrence	Police report required at this time	CPS/DCS report required at this time	Name of person filing report (s)
Physical	Yes	Victim	Childhood by father	No	No	
Emotional	Yes	Victim	Childhood by father	No	No	
Sexual	No					
Child	Yes	Victim	Childhood	No	No	
Elder/Depend ent Adult	No					
Domestic Violence	No					
Other types of Trauma/Histo ry	No					
Harm to Self or Others in a Treatment Setting	No					

Psychosocial Assessment

Flaherty, Dallas G

MR#: **109931**

Hosp #: **2501689**

DOB: **11/03/1994**

Admit Date: **07/22/2025**

Observation Date/Time:

Initial

Attempts made for completion (Attempts are made every shift until completed)

Start Time **7/23/2025 19:00**

End Time **6/23/2025 22:44**

Acute Symptoms **Depressed Mood/ Hopelessness, Feelings of Worthlessness, Inability to Concentrate, Suicidal Ideation w/Plan, Chemical Dependency, Family/Marital Discord, Homicidal/Danger to Others and Severe Anxiety/Panic Attack**

Chief Complaint (as stated by patient at Intake) **States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away."**

Chief Complaint (as stated by patient to clinician) **"My father is an inconsiderate prier, I was having an off day, and me and my father was bickering back and forth which escalated, and he was yelling because my uncle left some coins for the children and I called and asked for some money because my uncle has a blood clot in his lungs and the sale /distributions of the coins has been paused for about a year and a half. I also tried to get my mother to talk to him and she was not calling and or understanding what I'm saying which got me even more frustrated and I felt like I wanted to give them a straight punch. My mother called the ambulance, and I was sectioned"**

How is the Chief Complaint impacting activities of daily living? **"I can take care of myself, a few days ago I cook steak for my mother and self"**

Has the patient been in treatment previously? **No**

History and Current Risk Factors

History and Current Risk Factors

Legal Issues: **I have no legal issues**

Family/Social/Cultural/Spiritual

FAMILY INVOLVEMENT IN TREATMENT

Family Involvement in Treatment **Patient does not want family involvement**

ROI Obtained **Yes**

SOCIAL/PEER RELATIONSHIPS

Psychosocial Assessment

Flaherty, Dallas G

MR#: **109931**

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Observation Date/Time: **7/23/2025 7:31:53PM**

Describe current social functioning	"I had a few friends, but we grew apart"
Significantly different from past social functioning?	"Growing up I was constantly harassed bullied"
Who does patient identify as current support system?	Fiancé, mother
Has current illness/behavior affected patient's ability to utilize support system?	No

FAMILY HISTORY OF MENTAL ILLNESS

Family of Mental Illness	Yes
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Family hx of bipolar, anxiety, depression and SUD.

CURRENT MARITAL/FAMILY RELATIONSHIPS

Marital Status	Engaged
Are there current or past issues related to patient's relationship status?	No
Describe current relationship status, identify children and relevant family relationships	"I never get along with my father, but I somewhat get along with my mother and my fiancé"
Sexual Orientation	Hetrosexual

CULTURAL/ETHNIC DIVERSITY

Gender Identity	Male
Cultural Concerns	nothing noted
Special needs surrounding patient identity	nothing noted

SPIRITUAL

Identified spiritual support, believe in a higher power or have any religious involvement?	Yes
Religion	Protestant
Does the patient want clergy involvement?	Yes
Identified values and belief system	"I believe in God"

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Stated Leisure/Recreation Interests **Arts/Crafts, Music, Plants/Gardening, Reading, TV/Movies, Outdoor and "fishing, shoot pool, outdoors"**

Possible Leisure/Recreation Barriers **Social Skills, Financial and Communication**

Educational/Vocational Functioning

MILITARY HISTORY

Is the patient active or former military? **No**

EDUCATIONAL HISTORY

Highest Level of Education Completed **I left school at 10th grade**

Vocational Rehab Needed **Yes**

Has Job/School been impacted by current illness? **No**

Is patient currently in school? **No**

Learning Disability **Yes**

Explain **ADHD**

BARRIERS TO LEARNING

Barriers to Learning **Cognitive Capacity Deficits**

BEHAVIORAL ISSUES

Behavioral Issues **None**

Patient interview

Patient Interview

Mood/Affect **Optimistic**

Cognitive Functioning/Thought Process **Alert, Oriented and Attentive**

Patient's Strengths **Stable Housing, Supportive Family/Friends and Hx of Psychiatric Stability or Sobriety**

Psychosocial Assessment

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Patient's Limitations	Lack of Motivation, Unemployed and Perseverating on mom/ blame shifting
Patient admits to substance abuse	Yes
CD Referral	Yes
Current Medical Conditions	Patient Denies, "Bipolar disorder, current episode manic without psychotic features, severe, Autistic disorder, Generalized anxiety disorder, Personal history of traumatic brain injury, Attention-deficit hyperactivity disorder, unspecified type, Essential (primary) hypertension, Pervasive developmental disorder, unspecified, Presence of other specified devices" and Seizure d/o-last seizure was 2002 per pt.

Discharge Information/Recommendations

Discharge Information/Recommendations

Current Living Situation	Lives with Family
Can Patient Return	Yes
Identify Supports in Home	Parents and fiancé. New OP therapist appt booked on 8/8/25.
Income Source/Job Status	Food Stamps and SSI/SSA/SSD/SDI
Referrals	Outpatient and Primary care Physician
Support Involvement	Provide education for patient/family/significant others regarding MD/CD issues and Provide education for patient/family significant others regarding discharge plan/aftercare options which will promote recovery

Coordination of Care

Is patient or guardian willing and able to share information about their current outpatient providers including their PCP?	Yes
Has the patient's PCP been notified of their admission as appropriate?	No
	Pt denied having a PCP because he was trying a holist approach.
Initial Discharge Plan (Check all that apply)	Return to previous living arrangements and Outpatient psychiatrist

Psychosocial Assessment

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Post Discharge Goals(s) "patient's
own words"

"I want to go home and relax "

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Narrative Summary

Pt is a 30-year-old engaged English-speaking Caucasian male admitted to WBHH on 07/22/2025. Pt report that "My father is an inconsiderate pax, I was having an off day, and me and my father was bickering back and forth which escalated, and he was yelling because my uncle left some coins for the children and I called and asked for some money because my uncle has a blood clot in his lungs and the sale /distributions of the coins has been paused for about a year and a half. I also tried to get my mother to talk to him and she was not calling and or understanding what I am saying which got me even more frustrated and I felt like I wanted to give them a straight punch. My mother called the ambulance, and I was sectioned" Pt is diagnosed with Bipolar disorder, Autistic disorder, Generalized anxiety disorder, Personal history of traumatic brain injury, Alcohol abuse, Attention-deficit hyperactivity disorder, Cannabis abuse. Psychosocial stressors include lack of coping skills, emotion, depression, aggression, and his mental health issues.

Throughout the assessment, the patient was cooperative, appropriate, engaging, attentive, alert, and fully oriented to person, place, and time. He reports that he was born and raised in Massachusetts. He stated that he was primarily brought up by his mother, with whom he currently resides with his two cats. He reports a strained relationship with his father, who has a history of substance abuse, and identifies his father as his number one emotional trigger. He reports that he has three maternal half-siblings. He denies any history of legal involvement or exposure to domestic violence. He also denies any history of sexual abuse but endorse a history of physical, or emotional abuse and states that he has not experienced any trauma to his knowledge; however, he reports that at birth, he suffered a brain bleed due to the umbilical cord being wrapped around his neck, He also report that he was born dead. He identifies as both religious and spiritual and follows the Protestant faith. He reports that he is engaged to his fiancé and denies having any children. He states that he was bullied and ostracized in school, and notes that he was only able to complete up to the 10th grade. He acknowledges that he has learning disabilities but does not know their specific names. He denies hallucinations, both visual and auditory, and denies suicidal ideation or attempts. However, he does report an incident from his teenage years where he tried to choke himself with a belt. He reports that at times he can be impulsive and that he has difficulty with anger, which per ED record his mother described as constant since he stops taking his medication. He reports a history of past psychiatric hospitalization and acknowledges issues with sleep, though he states his appetite is good. He also reports that he takes medication to help him sleep. He reports that he has not been taking his psychiatric medication for about two months prior to admission, as he was attempting to follow a more holistic approach to managing his mental health. However, since being hospitalized, he reports that he has resumed taking his medication diligently. Per ED records his family members report that the patient has a history of verbal and aggression and threatening behaviors. The patient endorsed current homicidal ideation and stated that he had attempted to file a restraining order. He denies suicidal ideation currently. He reports a history of being pushed into the street by a stranger, which resulted in a back injury. He has lost a significant amount of weight, dropping from 260 to 202 pounds. He reports occasional alcohol use and social marijuana use. He also reports allergic to metoclopramide.

While at WBHH, Pt will likely benefit from medication intervention and monitoring to address negative symptoms of his mental health, group therapy for healthy coping skills development and identification of triggers, as well as referrals and aftercare

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planning to include follow up appointments for continuation of care and further recovery. Pt will be discharged once he has stabilized in medications, negative behaviors have resolved, negative symptoms of his mental health have improved or his WBHH team deems him appropriate to be discharge.

Patient agrees with Discharge Plan **Yes**

Family agrees with Discharge Plan **N/A**

Signature

I (staff) discussed the importance of participating in program milieu and participating in groups. **Yes

Therapy Staff Signature **e-Signed by Richards, Kevin at 07/23/2025 22:44**

Supervisor's Signature (if required) **e-Signed by Foster, Margaret, LICSW at 07/25/2025 08:01**

Abuse/Neglect/Trauma

Row	Yes/No	Victim/Perp	Documentation/T ime Frame of Occurrence	Police report required at this time	CPS/DCS report required at this time	Name of person filing report (s)
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Domestic Violence	No					
Other types of Trauma/Histo ry	No					
Harm to Self or Others in a Treatment Setting	No					

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Substance Use

Row	Used in last 30 Days	Age of 1st Use	Current Amount	Current Frequency	Current Duration	Date of Last Use/Amt	Route of Usage (IV, Smoking, etc.)
1	Nicotine	18	1.5 to 2 packs daily	Daily	12 years	Prior to ED Admittance	Inhalation
2	Cannabis	18	0.5 ounce daily	Daily	12 years	Prior to ED Admittance	Inhalation
3	ETOH	Unknown	Twice weekly: 6 beers/ 2 shots	Twice weekly	Unknown	Unknown	PO

Current outpatient providers including PCP

Row	Provider Name	Focus/Specialty/Role in patient wellness	Provider Contact Info	ROI Signed	Optional Notes
1	Alanya Bouchard	Therapist	508-663-3809	Yes	

Master Treatment Plan

Flaherty, Dallas G

MR#: 109931

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DOB: 11/03/1994

Admit Date: 07/22/2025

Observation Date/Time: 7/24/2025 5:09:48PM

Reason For Admission (In Patient's Words)

Dallas is a 30 y.o. male presenting to WBHH from Good Samaritan. Pt was sectioned by police, for HI (stating he was going to kill parents/ hit father) after an argument with his parents. PPHx of bipolar d/o, ASD, GAD, PDD nos, and ADHD. PMH of seizure d/o, TBI (brain bleed/ lack of oxygen at birth) and HTN. Allergy to Reglan. In-house urine tox positive for benzos and thc. Agitated in ED, required IM medications (Haldol and Ativan) both 7/21 and 7/22. Pt lives with Mom and wanted to put a restraining order against Mom, though both mom and pt are on the rental lease.

Pt arrived cooperative but labile, cycling between irritation, crying spells, and fatigue. Signed CV. VSS. States reason for admission as "It was just a big big misunderstanding. I talked to my father on the phone and I got pissed with him and pissed at my mother and I needed to get away." Pt confirms diagnoses and reports he had a VP shunt revision 20 years ago. ROI signed for mom (Terry) in order to get medication information/ appointment information, as pt did not know all of his meds or doses, stating "they expire so I don't take them and I didn't fill them. They are always expiring". Mom called for medications list. She states he is noncompliant and has been off meds for at least "a couple months." Home medications include buspirone 10mg bid, amlodipine 10mg, metoprolol (dose unknown), hydrochlorothiazide 25mg, omeprazole 20mg, fluoxetine 40mg, and lorazepam 1mg, divalproex 500mg, 0.5 tab PRN for acute anxiety or agitation and 1 tab hs. Daily 1.5 to 2 packs cigarette smoker. Daily 0.5 oz cannabis user. Twice weekly drinker (6 beers/ 2 shots). Pt has upcoming appt with new therapist on 8/8. Perseverating on mom being the cause/ reason he is here. Perseverating on appointment that he thinks he has on Friday, but mom reports his appt isn't until 8/8. Pt states "There she goes lying again. This is all her fault."

15-minute checks, low suicide precautions, AA and seizure precautions. Telephone orders from Dr Marks: Continue medications except metoprolol, decrease PRN lorazepam to 0.5mg bid. Start nicotine patch and gum PRN.

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Diagnosis Code	System	Class	Type	Priority	Date
Bipolar disorder, current F31.13	ICD-10	Psychiatric	Provisional	Primary Diagnosis	
episode manic without psychotic features, severe			1		
Autistic disorder F84.0	ICD-10	Psychiatric	Working		
Generalized anxiety disorder F41.1	ICD-10	Psychiatric	Working		
Personal history of traumatic Z87.820	ICD-10	Medical	Working		
brain injury					
Alcohol abuse, uncomplicated F10.10	ICD-10		Working		
Attention-deficit F90.9	ICD-10		Working		
hyperactivity disorder, unspecified type					
Cannabis abuse, uncomplicated F12.10	ICD-10		Working		
Epilepsy, unspecified, not G40.909	ICD-10		Working		
intractable, without status epilepticus					
Essential (primary) I10	ICD-10		Working		
hypertension					
Nicotine dependence, F17.200	ICD-10		Working		
unspecified, uncomplicated					
Pervasive developmental F84.9	ICD-10		Working		
disorder, unspecified					
Presence of other specified Z97.8	ICD-10		Working		
devices					

Admission Status

Conditional Voluntary

Master Treatment Plan

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Psychiatric

Problem - Danger To Others r/t Physical Aggression, Homicidal (HI statements towards parents, property destruction,) - Acute Care 07/22/2025

Objective - Patient will, prompted or independently, use a self-identified regulation skill to safely manage danger to others, homicidal ideation, and/or physical aggression. 07/22/2025 07/31/2025

Intervention - Conduct group (art, music, animal, movement, expressive) and monitor patient progress for the development of new leisure skills to decrease danger to others, homicidal ideation, and/or physical aggression TWICE A DAY 07/24/2025 Morton, Kristen, OT, MS, OTR/L 10912

Intervention - .Nursing will when Dallas identified with potential loss of control, ask Dallas what PRN medication is most helpful and administer medication according to doctor's order. TWICE A DAY 07/24/2025 Achang, Zainabu, RN BSN

Problem - Danger to Self (drinking -intoxication, punching objects) - Acute Care 07/22/2025

Objective - Patient will, prompted or independently, use a self-identified regulation skill to safely manage suicidal ideation and/or danger to self. 07/22/2025 07/31/2025

Medical

Problem - Altered Cardiac Output - Hypertension/Hypotension (Medication regimen, Verbal report of history of Hypertension) - Acute Care 07/22/2025

Goal - Dallas will be able to identify signs and symptoms of increased BP by time of discharge 07/22/2025 07/31/2025

Objective - Dallas will take medications as ordered 07/22/2025 07/31/2025

Intervention - Nursing staff to administer medication and treatment(s) as ordered per MD ONCE A DAY 07/23/2025 Avramidis, Angela

Problem - Nicotine Replacement/Smoking Cessation (Use of nicotine patch or gum per facility protocol, History of excessive nicotine use) - Acute Care 07/22/2025

Goal - Dallas will be compliant with nicotine replacement 07/22/2025 07/31/2025

Objective - Dallas will practice an identified coping skill to manage/reduce smoking urges 07/22/2025 07/31/2025

Intervention - Nursing will administer nicotine replacement protocol as ordered per MD As Needed 07/22/2025 Avramidis, Angela

Problem - Seizures (Alteration In Neurologic Homeostasis) (History of seizures. Last seizure in 2002 per pt. Pt noncompliant with medications, including seizure prevention meds.) - Acute Care 07/22/2025

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Goal - Dallas will follow up with primary care physician for continued issues related to seizures 07/22/2025 07/31/2025

Objective - Dallas will exhibit an understanding of their seizure disorder prior to discharge 07/22/2025 07/31/2025

Intervention - .Staff will monitor patient as ordered by physician EVERY SHIFT (8HRS) 07/22/2025 Avramidis, Angela

Strengths/Protective Factors	Ability To Communicate and Able To Meet Basic Needs
Limitations/Stressors	Angry/Agitated, Lack Of Insight To Illness, Marital/Family Conflict and Substance Abuse
Initial Discharge Plan	Attend PHP and/or IOP, Attend Recovery Group/Program, Individual Outpatient Therapy, Outpatient Psychiatrist, Primary Care Physician Follow-Up and Return To Previous Living Arrangements
Post-Discharge Goal(s)	"I want to go home and relax "

PATIENT INVOLVEMENT IN TREATMENT PLAN

The Treatment Plan Has Been Developed With Input From Me And I Have Been Given The Opportunity To Participate In Planning My Care During My Hospitalization.

Yes – With Individual Staff

PATIENT/GUARDIAN STATEMENT

The Treatment Plan Has Been Developed With Input From Me And I Have Been Given The Opportunity To Participate In Planning My Care During My Hospitalization.

Patient willing to sign? **Yes**

Patient Signature **Verbal Consent obtained from Flaherty, Dallas G at 07/24/2025 17:23**

Treatment Plan Signatures

Row	Signature	Credentials Or Relationship
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Master Treatment Plan

Flaherty, Dallas G

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Nursing Staff

e-Signed by Achang, Zainabu, RN BSN
at 07/25/2025 08:39

e-Signed by Achang,
Zainabu, RN BSN at
07/25/2025 08:39

Social Services

e-Signed by Campbell,
Deanna at 07/24/2025
17:24

Activity Therapy Staff

e-Signed by Morton,
Kristen, OT, MS, OTR/L
10912 at 07/25/2025
07:40

PHYSICIAN CERTIFICATION OF THE LEVEL OF CARE: I certify that this patient's inpatient psychiatric hospital admission is medically necessary for treatment which can reasonably be expected to improve the patient's condition and /or for diagnostic study.

Estimated Length of Stay: **7 Days**

Physician Signature **e-Signed by Okorafor, Ayodeji E, DNP at 07/25/2025 10:09**